



Healthy lifestyle promotion manual for youth leaders

Healthy lifestyle promotion manual for youth leaders

Getting started guide

Foreword

Each year, approximately 1.2 million children and youth die of Non-Communicable Diseases (NCDs) i.e. over 13 per cent of all NCD mortality as per WHO global status report on NCDs 2014. While some of these deaths are due to non-modifiable genetic factors, millions of children die of preventable causes each year. Additionally, children and adolescents exposed to modifiable NCD-related behaviours in youth are more likely to receive NCD diagnoses in adulthood. Persistent prenatal and childhood exposure to tobacco and alcohol, poor nutrition and lack of physical activity have long-term impacts on health and development, including increased risk of adult cardiovascular disease, diabetes and other NCDs throughout the life-course. NCDs affect more than physical health, often serving as a social determinant of reduced economic progress, increased poverty and potential for political instability. Prevention and management of NCDs is a long-term endeavour. Given that most NCDs are preventable, develop slowly, and are often lifestyle-related, suggested guidance and activities to eradicate them relate primarily to prevention and longer-term projects (two years or more) rather than to quick or short-term interventions.

In line with the International Federation of Red Cross and Red Crescent Societies' (IFRC) Framework for Noncommunicable Diseases and the IFRC Strategy 2020, the focus is on encouraging "action on the underlying social, behavioural and environmental factors that determine good health". The NCD vision of the IFRC for 2015–2020 is to reduce modifiable risk factors for NCDs and underlying social determinants by implementing community-based and population-based programming promoting healthy lifestyles aimed at primary and secondary prevention of NCDs.

The IFRC, with support of Danish Red Cross, has developed a set of tools that teachers, peer educators and youth group leaders can use in engaging youth worldwide in prevention of NCDs through reducing risk factors at the school and community levels. This NCD and Youth package contains a Getting Started Guide, a Peer Educator manual and a set of twenty lesson plans with high colour tools to be used in classrooms, youth groups, and any setting where youth live, work and play. This package is designed to assist those who work with youth to lead prevention activities to help influence youth, our most effective community change agents, in understanding, applying and practicing healthier behaviours as part of existing Red Cross Red Crescent health and youth programme and project plans at the local and national levels. This package of tools can assist those who work with youth by educating children and youth in preventing and/or controlling NCDs with activities to address the needs of children and youth at the individual, family and community levels. We hope that this guide and the accompanying toolkit will support those who work with youth in Red Cross and Red Crescent National Societies in the important and valuable work they are doing in their youth communities.



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Getting started guide

for healthy lifestyle promotion manual
and toolkit for youth

Acknowledgments

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Contents of the materials in the Healthy lifestyle promotion manual and toolkit for youth

- Getting started guide
 - 20 lesson plans and tools targeting youth and peer educators
 - Peer educator guidance
-

Learning objectives

As a result of the Healthy Lifestyles for YOUTh, participants will be better able to:

1. Demonstrate effective communication skills.
2. Identify the four NCDs of cancer, diabetes, cardiovascular disease (CVD) and chronic respiratory diseases (CRD).
3. Identify the four risk factors for NCDs.
4. Identify how they can consistently engage in healthy lifestyle behaviours.
5. Use appropriate techniques to positively influence peers.
6. Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
7. Assess health behaviours and determine recommended actions.
8. Explain the importance of supportive environments for healthy living.

Audience for Healthy lifestyle promotion manual and toolkit

Healthy lifestyle promotion manual and toolkit for youth includes 20 session plans for use by Red Cross Red Crescent Youth volunteers and staff to teach about healthy lifestyles to youth in the school and community. The activities included allow volunteers and youth to learn the four principal NCDs, the dangers of NCDs, the behaviours that contribute to NCDs, the basics of behaviour change, assessing their NCD risk and developing a supportive environment to be able to implement the guidance given. These session plans build on the good work of the YABC (Youth as Agents of Behaviour Change) project, aiming to empower youth to change the unhealthy behaviour of themselves and their peers. These outcomes can be achieved with the highly interactive tools included in this toolkit. A breakdown of the suggested time required for the sessions is displayed in the lesson plan map table below.

Users of this material may include teachers in a school setting, Red Cross Red Crescent youth volunteers teaching the content peer-to-peer or to younger children. These materials are written as useable lessons for audiences aged 12–17. Refer to the “Audience” column in the lesson plan map to identify audiences for which each lesson plan is appropriate.

- YPE = Youth peer educators
 - Y = Youth
 - A = Adults
 - a for A = can adapt for adults
 - a for YC = can adapt for younger children
-

Adapting materials

To adapt materials for other audiences, the process requires:

- Active collaboration with the target audience to determine areas needing adaptation
- Propose modifications
- Test materials and approaches
- Revise and monitor

Getting started guide

To adapt materials for age, culture, gender and/or literacy level, a team would start by reviewing existing material with adequate representation from the target audience. If there is more than one group (culture, sex, age, religion) for which material needs to be adapted, it is critical that at least two representatives from each type of group be part of the adaptation process. If adapting for very young children, invite early childhood teachers to join the group to represent appropriateness for young children.

To start:

- Provide hard copies of the specific lesson plans that might be adapted.
- Provide a five minute session to simply define the four risk factors of unhealthy diet, physical inactivity, tobacco use and excess alcohol.

Active collaboration with the target audience to determine areas needing adaptation

- Either teach the lesson plan as written OR ask the group to read once through the material on their own.
- Ask:
 - ◇ What is your general reaction to this draft?
 - ◇ For whom do you think this material or product is meant?
 - ◇ Which material do you think is easiest to understand?
 - ◇ Which material do you think shows a situation closest to your life?
 - ◇ Which material presents the most believable message?
 - ◇ If you were to use this material to teach to your culture/age group/gender/religion, which material and/or methods would you like to keep?
 - ◇ If you were to use this material to teach to your culture/age group/gender/religion, which material and/or methods would you get rid of?
- Give each individual a marker and sticky notes. Individuals representing a unique culture/age group/gender/religion group may work together. Instruct the group to review the material and highlight the text, images and/or methods that do not apply or are inappropriate to their culture/age group/gender/religion. They should place a sticky note on top and write alternative text or draw an alternative image that could be used. If the method is inappropriate, they may suggest a more suitable method.

Propose modifications

- Using the inputs from the teams, work together to revise the materials per their suggestions. If appropriate, allow the target group to work with the team to make the edits for their adaptation.

Test materials and approaches

- Using the adapted materials, test all materials with a sample of the target group. They should be used by a facilitator familiar with or from that same culture/age group/gender/religion.
- After each section of material, ask the audience:
 - ◊ What did you learn?
 - ◊ What did you like?
 - ◊ What did you not like?
 - ◊ What would you change in the materials or the method?
 - ◊ After the testing, query the facilitator using the same questions.

Revise and monitor

- Revise the materials using the feedback received. If appropriate, ask the facilitator to help revise the materials to a final version.
 - Monitor how the target group responds to the materials. Revise again if needed
-

Where do I start?

The lesson plan map below details the learning objectives for the Healthy lifestyle promotion manual and toolkit for youth. These lesson plans can be taught sequentially or according to the knowledge and skills needed for a specific lesson or session. The toolkit is designed to align with school curriculum specifically in the areas of:

- Health education on proper nutrition, smoking and alcohol
- Advocating proper nutrition – canteen, sari-sari stores surrounding the schools to cease selling cigarettes and alcohol to minors
- Advocating against smoking and drinking alcoholic beverages
- Dissemination of advocacy messages and materials for and between students, parents, school personnel and community

Lesson plan map			1. Identify the four NCDs of cancer, diabetes, CVD and CRD	2. Identify the four risk factors for NCDs
	Audience	Time		
Behaviour change DOs&DON'Ts	YPE; Y; a for A	30-45 min		
Behaviour change stages	YPE; Y; a for A	45 min		
Collaborative negotiation	YPE; Y; a for A	90 min		
Effective communication	YPE; Y	60 min		
Environmental scan	YPE; Y; a for A	75 min		
Gender in healthy lifestyles	YPE; Y; a for A	90 min		
Healthy foods debate	YPE; Y	90 min		
Healthy lifestyle game show	YPE; Y; A	60 min		
Healthy lifestyle relay	YPE; Y; a for YC	45-60 min		
Name that activity	YPE; Y; a for YC	30-40 min		
Name that risk factor	YPE; Y; a for A	45-60 min		
Supportive environments	YPE; Y; a for A	60 min		
Peer pressure	YPE; Y	45 min		
Personal resilience	YPE; Y; a for A	90 min		
Playing my role	YPE; Y; a for A	90 min		
Project artwork	YPE; Y	120 min		
Someone else's shoes	YPE; Y	45 min		
Special effects	YPE; Y; a for A	45 min		
Telephone	YPE; Y; A	30-40 min		
Action plan template	YPE; Y; A	20-30 min	The action plan is to be used at the end of every lesson.	

It is a method for summing up the learning and holding learners accountable for applying the content to healthy change in their own lives until the next time they meet with the Red Cross Red Crescent youth members.

Time required

The materials in this toolkit are flexible and are not necessarily designed to be taught in sequential order. Content from one activity does not always build on other activities UNLESS specifically indicated on the activity guidance. Each lesson plan in this toolkit ranges from 45–120 minutes required for the full experience. The times for each individual lesson are listed in the lesson plan map table above.

How do we know that the materials are working?

The lessons are designed to teach knowledge, skills and behaviours and the assessment of learning is integrated into the design of the lessons. Read each of the learning objectives featured in each individual lesson plan – these are the observable actions the facilitator or trainer should be able to see each learner actually demonstrate. For example, for the lessons with the learning objective of “Demonstrate effective communication skills”, there is an opportunity to actually demonstrate effective communication skills. This is something that can be physically observed. By teaching all of the lesson plans with this same learning objective, the learning objective of demonstrating effective communication skills becomes concretized and retained in the learner’s mind and skillset. There are many lesson plans that contribute to each learning objective to allow the learner to observe and demonstrate the complexities of the knowledge, skill and behaviour required to achieve that learning objective. Learning is layered on top of previous learning to improve the likelihood of retaining that content.



Peer educator guide

for healthy lifestyle promotion manual
and toolkit for youth

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Audience

This guide has been developed for use by peer educators in Red Cross Red Crescent youth programmes. It is not meant to be an exhaustive list of dos and don'ts but rather to provide you, as a peer educator, with the information you need to start a peer education group, together with information about behaviour change practices of which you should be aware to lead youth towards a healthy lifestyle. Youth peer educators are an important part of any National Society's defence against noncommunicable disease (NCD) risk. This guide will help you become a more informed agent of behaviour change in your community.

Learning objectives

A peer is a person who belongs to the same social group as another person or group. The social group may be based on age, sex, sexual orientation, occupation, socio-economic or health status and other factors.

Education refers to the development of a person's knowledge, attitudes, beliefs, or behaviour as a result of the learning process.

Peer education can cover any area such as health promotion, community development or whatever is needed in a specific community. Peer educators in the Red Cross Red Crescent are volunteers who help to keep an equal relationship between you as a coach or teacher and people your age who are learning new knowledge, skills and behaviours. Peer education is the most effective way to improve community health by spreading needed information and skills throughout the community. A Red Cross Red Crescent youth peer educator is one of the most important people in their community of peers as they can help people gain knowledge and skills to improve their lives, become more resilient, advocate for themselves and their needs and have more control over their own health.

Peer education can take place in small groups or through individual contact and can happen in any number of settings: schools, universities, clubs, religious institutions, workplaces, street settings, shelters, or wherever people of your peer group gather.

Examples of youth peer education activities might include:

- Organized sessions with students in a secondary school, where peer educators might use interactive techniques such as game show quizzes, role plays or stories
- A theatrical play in a youth club, followed by group discussion
- Informal conversations with young people at a Red Cross Red Crescent youth club, where they might talk about different types of behaviour that could put their health at risk and where they can find more information and practical help

Why peer educators?

Peer educators are considered more credible by people within their same age or cultural group. Youth who have taken part in health peer education sessions and initiatives say that they understand and can apply information they learn more easily. There is a common understanding between the peer educator and their audience because everyone shares a similar background and interests like music, pop culture, language or slang, common concerns or issues people of their age group face such as struggling for independence and the demands of being a good student and son/daughter.

Youth peer educators are less likely to be seen as authority figures “preaching” about how others should behave. Peer education feels to youth like they are receiving advice from a friend who can teach and help them and who has similar concerns and an understanding of what it is like to be a young person. A young adult teacher leading her 16-year-old students in a session about NCDs is not a peer educator since she has authority over those she is teaching. A young adult leading other 16-year-old youth in a session about healthy lifestyles and who has no authority over them IS a peer educator. Both of those young adults may have the respect of the group, but the authority is the difference. Do not attempt to tell people what to do or that they must listen to you. Your role is to be a coach and a support for healthy change.

Peer education can also empower young people. As a peer educator, you have the unique opportunity to lead and participate in activities that affect you and people of your age group to access the information and services you need to protect your health and the health of your friends and family members.

Peer educator objectives

The main objectives for being an effective peer educator include:

- To teach and coach your peers in the necessary knowledge that they need to make healthier choices to maintain a healthy lifestyle
- To teach and coach your peers in developing the necessary skills needed for a healthy lifestyle
- To support your peers in reinforcing positive healthy behaviours
- To support your peers in developing new recommended healthy behaviours
- To support your peers in identifying and changing their risky behaviours

What is the best way to achieve these objectives

While youth report that they like to receive coaching or teaching from someone in their peer group, it is also good for peer educators to understand how to be an effective peer educator. The best way to ensure that what you propose to do in leading a group with unhealthy lifestyles is to base your work and theories on practices that have been proven to be most effective for behaviour change. The science of why and how people adopt new behaviours or eliminate unhealthy behaviours is not exact. However, researchers in the fields of health psychology, health education and public health provide a solid evidence-based foundation to show how and why peer education is our most effective defence against NCDs. As a Red Cross Red Crescent youth volunteer, knowing these theories may help you when you are planning and implementing peer education sessions on healthy lifestyle. Let us take a look at some of the theories and models of behaviour change that are aligned with peer education.

A. The theory of reasoned action

The theory of reasoned action says that the intention of a person to adopt a recommended behaviour is determined by two things:

1. What that person believes about the new behaviour AND how they think the effects of this new behaviour will improve their lives. For example, a girl who thinks that eating more fruits and vegetables will help her lose weight and be more attractive will have a positive attitude towards healthier eating.
2. If the people important to the person approve or disapprove of the new behaviour. In our example, if the girl's friends refuse to give up fast food and eat healthier foods themselves, the girl is less likely to maintain her positive attitude towards healthy eating.

Young people's attitudes are highly influenced by their perception of what their peers do and think.

How can a peer educator influence their peers within the theory of reasoned action?

If you are a respected peer educator, your peers will be very much motivated by your expectations regarding healthy lifestyle choices. Understanding that people make healthy changes based on how they think it will improve them and also if their peers approve of the new behaviour gives you the power to help redefine how healthy lifestyles choices are cool or more acceptable by your peer group. In our example, a girl wants to make a healthy change but she fears she will lose the approval of her friends. As a peer educator, you can personally invite her friends to join your group and talk to them individually about how a healthy lifestyle can help them, their friends and their family.

B. The social learning theory

The social learning theory says that people learn in three ways:

1. Through direct experience. For example, a group of girls take a healthy cooking class and learn how to make healthy substitutes in recipes for their favourite foods.
2. Indirectly, by observing and modelling the behaviour of others with whom the person identifies. For example, a group of young men will be dressed similarly, use similar slang, listen to similar music and behave in similar ways. They teach each other and set the standards for what is acceptable behaviour in their group.
3. Through training or coaching to build their confidence that they can do the healthy behaviour. For example, a peer educator uses role plays to allow teens to practise how they can say no to a friend offering them a cigarette or pressuring them to smoke.

How can a peer educator influence their peers within the social learning theory?

Teach or coach peers by using lessons or sessions that effectively place learners in settings where they can practise healthy behaviour and practise saying no. The people or influences that try to disrupt their healthy behaviour are critical. Lesson plans for sessions that are most effective within this theory involve role plays, debates, simulations and any experience where a learner practises how they will adopt the behaviour, respond to negative influences, etc. Additionally, peer educators serve as very influential teachers and role models.

This does not mean that you must be perfect at the healthy behaviour being proposed, but that you are also trying to practise it on a daily basis.

C. The theory of diffusion of innovations

The theory of diffusion of innovations states that social influence plays an important role in behaviour change. Important leaders – even celebrities – in any community, school, family or peer group are agents for behaviour change. What a leader thinks or feels about a specific behaviour has tremendous impact on whether people in the community will adopt the behaviour. For example, Philippines-born Anne Curtis is a celebrity seen by youth in the country as a role model. Miss Curtis participated in the Philippines Department of Health's "OMG¹ Eat Your Vegetables Campaign" which aimed to "make vegetables and fruits sexy".



¹Oh my gulay: tagalog for "Oh my veggies".

How can a peer educator influence their peers within the theory of diffusion of innovations?

As a peer educator, you will automatically be perceived as a leader in your peer group. This role includes great responsibility to be trustworthy and credible with your peer group to provide them accurate knowledge and skills and to serve as a role model. Depending on where you are volunteering, the peers in your Red Cross Red Crescent youth group may not otherwise receive information and support about healthy lifestyle and behaviour change - making you their sole source of information.

D. The theory of participatory education

The theory of participatory education says that developing a group of peers talking among themselves and determining how they can make healthy lifestyle changes and choices both individually and as a group is key to success and lasting behaviour change. For example, after leading a peer group in an environmental scan around their school, the peer educator helps the youth to see that there are no healthy food options in the direct vicinity of the school. The peer educator discusses with the group how they can get vendors and shops to sell healthy snacks in the near neighbourhood, and the group takes action to make it happen.

How can a peer educator influence their peers within the theory of participatory education?

By helping your group to recognize or see opportunities available to them for healthy lifestyles and allowing them to discuss options, you are leading them in participatory education. Your role within this theory is not to lead the group, but to facilitate their discovery of the necessary knowledge, skills and behaviour and what they can do to address issues that they themselves raise. You can then lead them in discovering ways to address issues that have been raised and mobilize them into action.

E. The health belief model

The health belief model says that if a person wants to avoid NCDs (or they want to feel better if they have an NCD) and they believe that a specific health behaviour will prevent NCDs, then they will do the positive behaviour. Importantly, this model also addresses obstacles that a person feels may prevent them from doing the healthy behaviour and if it is worth it to do the healthy behaviour. For example, if a boy in your peer group does not stop chewing tobacco because he believes that it looks cool and is safer than cigarettes, you can provide information about how chewing tobacco affects the body and contributes to cancer risk. You can also show pictures of people he looks up to - such as celebrities or sports figures - and discuss how they are not chewing tobacco because of the health risks.

How can a peer educator influence their peers within the health belief model?

Peer educators can focus on how they might reduce what their peers may consider to be barriers to healthy living by reassuring peers that they can do it, correcting any misinformation, providing incentives to start and maintain healthy behaviour and support for continued maintenance.

F. The social ecological model

The social ecological model for health promotion says that behaviour is determined by:

- Intrapersonal factors like our own knowledge, attitudes, behaviour, self-concept and skills
- Interpersonal processes like our social networks and social support systems, including our family and friends
- Social institutions like Red Cross Red Crescent, school and other places where we interact with others
- Community factors like where we live, where we work and where we play
- Public policies which affect us such as local, state and national laws and policies

We interact across all of these levels and each has an influence on our choices of healthy and unhealthy behaviours.

How can a peer educator influence their peers within the social ecological model?

As a peer educator, it is important to recognize that peer education is just one part of the influence on your peers' behaviour choices. While peer education is often considered the most important intervention to influence healthy lifestyle change, you should be sure to coordinate your groups' activities with other activities hosted by schools, communities and public policy. Your role as peer educator is to align your efforts with activities that support your work within the peer community.

Putting these theories into practice

"Tell me ... I forget, show me ... I remember, involve me ... I learn."

-Benjamin Franklin

"Having heard something is not as good as having seen it. Having seen it is not as good as knowing it. Knowing it is not as good as putting it into practice."

-Xun Kuang

As a peer educator, you are acting less like a teacher and more as a facilitator of learning. This means that both peer educator and the learners understand the peer educator's role is to provide information, skills and motivation while the learner's job is to apply it towards a healthier lifestyle. It is most important to remember the peer educators do not need to know all the answers all the time but can create a powerful learning experience by often learning alongside the learner in helping the group to find the answer.

Methodologies in the healthy lifestyle programme session plans for youth attempt to translate relevant behaviour change theory into practice. The session plans are written for an audience of 12 to 17 years of age with the assumption of a peer educator leading the session. Learning is based on experience, observation, skills practice and experiential practice and adopting healthier behaviours through the use of interactive methodologies.

By using these interactive methodologies, you involve participants in an active way that incorporates their own understanding and experience. This gives your trainees opportunities to develop the knowledge and skills they need.

So how exactly do I start a group where I can serve as a peer educator?

As a peer educator, you will coordinate with your Red Cross Red Crescent supervisor and discuss potential sites for people of your age group who are at most risk. Your National Society branch has partnerships with important places like schools, after school groups, youth groups, etc. You and the National Society youth team need to identify appropriate sites where people of your age group and/or culture gather to work, live and/or play.

By securing meeting space and the necessary approvals to hold regular Red Cross Red Crescent youth group meetings, you are on your way to starting a healthy lifestyles group. Once you have advertised the start-up of your youth group, you may need to go out and solicit your first initial members. Start with your friends or family or classmates and invite people to join you.

What do I do once I have invited people to my peer session?

Red Cross Red Crescent has many resources that you can use in teaching youth. These materials are available from your National Society branch. You will see that there are many materials available. Refer to the implementation guidance for these materials on how to assess your group and develop a plan for what you will do in your regular sessions or meetings. The lesson plans in this toolkit provide all materials needed and step-by-step guidance for how to lead your group. Each lesson plan also provides learning objectives towards which that lesson plan contributes. This is not to mean that one lesson plan will change everyone's behaviour, because behaviour change is a slow and gradual process that requires regular application of knowledge, skills and behaviours.

When should I modify my peer education plan or teaching materials?

Follow your plan that you develop from the implementation guidance. But always be comfortable in modifying your plan based on your group's needs and how well they respond to material you have previously presented. For example, if you are teaching a session about NCDs and the session reveals that there are several people in your group with a diagnosed NCD, you will have to modify future lessons to accommodate and respond to the needs of these individuals. For example, in a role play session aimed at addressing misinformation about NCDs, you may ask those individuals with NCDs to contribute misinformation they have heard to the discussion to allow learners to better understand the complexity involved. Remember that as a peer educator, you are a facilitator of learning which means that you lead the group discussions and activities towards greater understanding of healthy lifestyle knowing that you do not have all the answers. Facilitators help to pull information and beliefs and feelings from learners to help them process new information and strategies.

How long do I “do” peer education?

Remember that peer education relies on the person leading or facilitating to be within the same peer group as those being led or facilitated. This means that you can be a peer educator throughout your whole life, but the age group you are leading or facilitating will continually change over time.

There are many lesson plans and resources from the IFRC for you to conduct peer education sessions on healthy lifestyle. These are just a resource for you - when you complete all the lesson plans it does not mean that healthy lifestyle support groups need to end. You are welcome to find other lesson plans and resources to use with your peer education group and you may continue indefinitely, with the support of your Red Cross Red Crescent youth supervisor.



Lesson plan guidance

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Behavior change stages

Time required

45 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

Learners learn that behaviour change is difficult and that it is fine to struggle, as long as they keep trying. They learn about behaviour change through exploring how one example person struggles with behaviour change and then they play a game to show how to be patient with the process and how to support others.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Assess health behaviours and determine recommended actions.
- Explain the importance of supportive environments for healthy living.

Materials

- Behaviour change matching cards
- Slides and steps game board
- Coins (one for every two participants)
- Pawns (one for every participant)

Preparation

- Print out one copy of the behaviour change matching cards for every five participants. Do NOT print double-sided. Cut the cards into ten individual cards and clip together or place in envelopes.
- Print one copy of the slides and steps game board for every five participants.
- Gather one coin for every five participants
- Collect five pawns for each group. These can be coloured paperclips, balled-up sticky notes, stones, anything easy to find.

Process

1. Explain that the best way to lower your risk of NCDs is usually to change some unhealthy behaviour or start doing a healthy behaviour.
2. Explain that there are five general stages in behaviour change. The steps are: knowledge, approval, intention, practice and advocacy. People move up and down between these stages frequently for all sorts of reasons.

To change a behaviour, a person must have a clear goal, know what their motivation is to change and have support. Behaviour change requires a daily commitment to healthy behaviours.

In each person, every behaviour is triggered or prompted by specific people, events or activities. Understanding these triggers helps people to be prepared to practise healthy behaviours when the triggers happen.

3. Explain that they will learn more about these stages through a matching activity. Ask participants to get into groups of four to five. Provide a slides and steps matching packet to each group.

Each packet contains ten cards of which five are a slightly different colour. Their goal is to place the coloured cards in order based on the stages of behaviour change. Next to each stage, they should place the example card about Benny which illustrates that specific behaviour change stage.

4. When everyone is clear on the directions, ask them to begin.
5. After two to three minutes or when everyone has completed the task, ask how they did. Review each stage by asking someone to read the coloured card for that stage. Then ask what was happening with Benny in that stage by reading the white card with the example.
6. Remind them that as a peer educator, their role is to help their community at home and school get and stay in the "practice" stage and not fail. To support them:
 - Remind them often of their goal.
 - Remind them often of their motivation for reaching the goal.
 - Give them clear guidance on how to reach their goal.
 - Give them frequent reminders of how to successfully respond to challenges or triggers in their environment.
 - Provide support from others going through similar changes in behaviour, such as in a community group.

Lesson plan guidance

Behavior change stages

7. **Clarify that the practice stage is just that – practice.** No-one is perfect – even peer educators can slip in the stages or can occasionally have an unhealthy behaviour. That's fine. Explain that all Red Cross Red Crescent youth can expect is the continued practise of healthy behaviours. Soon, they will become habit.

8. Explain that they should stay in their groups to play a game called slides and steps which is somewhat similar to the game of snakes and ladders.

Explain that behaviour change is very difficult. Even when people want to only do their best, they may struggle to maintain changes at first.

Give a slides and steps game board, a coin and five pawns to each group.

9. Explain that behaviour change can take time and patience. The purpose of this game is to reach the last square before anyone else. Each of the squares on the game board features a behaviour change step or stage.

They should place their pawns behind the finish line. The oldest person goes first by flipping the coin – if they flip heads, they move forward one. If they flip tails, they move forward two. When they land on a spot, they will see the stage of behaviour change they have reached.

If they land on a space with a ladder, they may advance to the stage in the box where the top of the ladder ends. If they land on a space with a slide, they must slide down to the stage in the box where the bottom of the slide ends.

10. If everyone is clear on the directions, let them begin.

Once most of the teams have finished, call everyone to plenary to discuss. Questions to include may be:

- a. How many of you want to change a behaviour and are still only at the approval stage?
- b. How many of you want to change a behaviour and are still only at the intention stage?
- c. How many of you want to change a behaviour and are still at the practice stage? How do you stay at that stage? What have you done to make this easier for you to keep practising?
- d. How many of you want to change a behaviour and are at the advocacy stage?
- e. Do you think it is possible to be a peer educator if you are not at the advocacy stage with any one unhealthy behaviour? Explain.
- f. If you are practising an unhealthy behaviour regularly, how does that make being a peer educator more difficult?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

"There are many interventions that can be successful in helping people to adopt healthier behaviours. These interventions are different for different contexts and determinants. There are five general stages in behaviour change. The steps are: knowledge, approval, intention, practice and advocacy. People move up and down between these stages frequently for any number of personal reasons. Behaviour change is not an exact science. Behaviour change requires a daily commitment to healthy behaviours.

The three things to consider when changing behaviour understands a person's motivation to change, the clarity of their plan in reaching their goal and ensuring that the person's environment supports the change. In every person, behaviour patterns are triggered or prompted by specific people, events or activities. Understanding these triggers helps people to be prepared to practise healthy behaviours when the triggers happen."

Source: http://www.ifrc.org/Global/Photos/secretariat/201402/20140228-NCDs-Toolkit/Volunteer_Manual_MRC_02262014.pdf

Behavior change dos and don'ts

Time required

30 – 45 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

Learners play a game in groups of five participants to learn the consequences of unhealthy behaviour and the rewards of making healthy choices. Fun!

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.

Materials

For every five participants – one healthy life game packet which includes:

- Game board
- Dice (buy these locally)
- Game pawns – use something locally – paper clips, stones, coins, etc.
- 45 healthy life game cards
- Healthy life game Instructions

Preparation

Prepare one healthy life game packet for every group of five. For each game packet:

- Print one game board.
- Print one copy of healthy life game cards - 47 in total (Note to facilitator that the two pink game cards refer to drinking alcohol – you may include these or remove depending on context and audience)
- Print one copy of healthy life game instructions
- Get a die and five player pawns.
- Place all materials for one packet into a large manila envelope.

Process

1. Introduce the learning objectives for the session and give a brief overview of the activities in this session.
2. Explain that there are two general types of behaviour change: Dos – healthy habits that we want to start doing and Don'ts – unhealthy habits we want to stop doing.

DO behaviours are healthy habits that we want to **start** doing such as eating more fruits and vegetables, or doing some physical activity. **These are activities that we now have to make time for within our day or routine.** These are easy to forget or ignore, simply because they are not yet part of your routine. These are often behaviours that we keep meaning to do, but somehow never quite do.

DON'T behaviours are unhealthy habits that we want to stop doing such as stop smoking or stop drinking or stop worrying. **These are activities which we regularly do and which have become comfortable for us to continue doing, despite any troubles we may feel as a result.** For example, a person who knows that they should stop eating 10 candy bars every day because it is unhealthy and because they are becoming obese will find it hard to stop – this has become part of their everyday activity. **To help yourself stop a DON'T behaviour, you should replace the unhealthy action with a healthy or neutral action to help your brain build a new pathway or habit.** When you feel the need for candy bars, ask yourself if you are really hungry. If so, eat an apple. If not, replace the habit of candy bars with another habit like walking around your house or doing a simple stretch at your chair or calling a friend. After several days, your brain will start to recognize the new path(s) and it becomes a little bit easier each time to say no.

3. Explain that they will have a chance to play a healthy life game to understand these different behaviours and how our choices help us or hurt us and how they can teach others how to live healthier lives.
4. Open a healthy life game packet and show the contents. Briefly explain the directions and tell them that the goal of the game is to be the first person to reach the finish line.
5. Ask participants to get into groups of five people. Give one healthy life game packet to each group.

Lesson plan guidance

Behavior change dos and don'ts

6. After the game finishes, have a full group discussion to talk through what they are feeling after the game. Questions can include:
 - How did you find the game?
 - What was the most frustrating part of the game?
 - What was interesting in the game?
 - Which DO behaviours did you find?
 - Which DON'T behaviours did you find?
 - Did you find that helping others allowed you to move further forward or did it have no effect? Why do you think this was?

Wrap-up

Review the session objectives. Answer any remaining questions.

Colaborative negotiation

Time required

90 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

This adaptation of a YABC lesson allows learners to role-play different scenes between a youth and someone to whom they look up, such as a peer educator. Learners are placed in situations where they are either willing or unwilling to compromise and will experience how difficult it can be to find a solution that meets each person's needs in these different scenarios. Learners thereby explore the impact of mindsets, attitudes and behaviours related to standoff and one-sided compromise to finally reflect on methods and techniques that help to build and achieve consensus and mutually satisfactory solutions.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Identify the four risk factors for NCDs.
- Use appropriate techniques to positively influence peers.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Assess health behaviours and determine recommended actions.
- Explain the importance of supportive environments for healthy living.

Materials

- STANDOFF, COMPROMISE and CONSENSUS signs
- Chalk, tape or string

Preparation

- Print one copy of the STANDOFF, COMPROMISE and CONSENSUS signs
- Make a line with chalk, tape or string on the ground from one end of the room to the other.
- Write or place the STANDOFF sign on one side of the line and the COMPROMISE sign on the other side. Write or place the CONSENSUS sign in the middle.

Process

1. Introduce the session, explaining to participants that youth are proven to be the most effective agents of behavioural change.
2. Explain that they will take part in different negotiation scenarios to help them understand how they can inspire and influence others' way of thinking and behaving for healthier lifestyles.
3. Ask participants to form pairs.
4. Explain that each pair represents two people. One person will be a teenage niece or nephew while the other person will be their slightly older uncle or aunt with whom they are currently living. Each pair should decide on who will play which role.

First briefing

5. Ask all the people playing the uncle or aunt to come to the side of the room for a briefing. Explain to the group that their niece or nephew (their pair's partner) is coming to them to talk about something which the uncle or aunt feels is a tremendous risk. Their niece or nephew want permission to do this thing. Clearly specify that, at this stage, they stay absolutely determined and unwilling to move from their position. They should firmly disagree and refuse to consider their niece or nephew's request. When everyone understands, ask them to be seated.
6. Ask all the people playing the niece or nephew to come to the side of the room for a briefing. Explain to the group that they are playing a teenager who suddenly has the opportunity of a lifetime to do something they have always wanted to do. This might be travelling around the world, studying abroad, acting in a movie, whatever. For that dream to come true, their uncle or aunt has to give permission. Clearly specify that, at this stage, they are so very determined to achieve what they want that they are absolutely unwilling to move from their position. When everyone is clear, ask them to be seated.

First pairs negotiation

7. Once all participants can clearly visualize the situation, their characters and roles, invite the niece or nephew to start a conversation with their respective uncle or aunt and engage in the role play whenever they feel ready to do so.
8. After a few minutes, stop the negotiation processes.

First process mapping

9. Ask all participants to sit on the side of the tape / line on the ground (standoff, compromise or consensus) which they think corresponds to the current situation they have just role-played.

First plenary

10. Hold a short plenary:

- Nieces/nephews:
 - ◊ How did you feel about your character? How did you feel acting out this role?
 - ◊ Did you get what you wanted?
 - ◊ Was your relationship with your uncle or aunt affected? How and why?
 - ◊ Were you willing to compromise?
- Uncles/aunts:
 - ◊ How did you feel about your character? How did you feel acting out this role?
 - ◊ Was your relationship with your niece or nephew affected? How and why?
 - ◊ Were you willing to compromise?
- Ask full group:
 - ◊ How would the negotiation process have ended if one of you have been willing to move from his or her position? Why?
 - ◊ What different techniques were used that helped the situation?

Second briefing

11. Ask all the uncles and aunts to stand up. Ask them to count off by twos. The number twos should then come to you for a private meeting while all the number ones stay standing.

In your private meeting, tell the number two uncles and aunts that they should stay in disagreement with their niece or nephew but this time, they should be willing to make a compromise. Ask them to go back to the original place where they started the negotiations with their niece or nephew.

Then ask the number one uncles and aunts to come to you for a private meeting. Tell them that they should remain completely stubborn and not willing at all to move from their position or compromise on anything. They should refuse any request or proposal from their niece or nephew. Once they are clear, ask them to go back to the original place where they started the negotiations with their niece or nephew.

12. Now ask the nieces and nephews to stand up. Ask them to count off by twos and then ask all the ones to come to you for a private meeting while all the twos stay standing. The number one nieces and nephews are expected to be those forming pairs with the number one uncles and aunts in order to make sure that in each pair there is one person willing to compromise and another being stubborn.

In your private meeting, tell the number one nieces and nephews that they should stay determined to realize their dreams and get what they want, but this time are also willing to make a compromise. Ask them to go back to the original place where their uncle or aunt is waiting for them.

Ask the number two nieces and nephews to come to you for a private meeting. Tell them they should remain completely stubborn and absolutely determined to get what they want with no compromises. Ask them to go back to the original place where their uncle or aunt is waiting for them.

Second pairs negotiation

13. All the pairs should now be reunited. Make sure that all directions are clear. Once they understand, they can resume their discussions.

Second process mapping

14. After a few minutes, stop the negotiation processes and ask all participants to be seated on the side of the tape lying on the ground (standoff, compromise or consensus) which they think corresponds to the situation they have just role-played.

Second plenary

15. Hold a short plenary:

- Nieces/nephews:
 - ◊ How did you feel acting out the role this time?
 - ◊ Did you get what you wanted this time?
 - ◊ Was your relationship with your uncle or aunt affected this time? How and why?
 - ◊ Were you willing to compromise?
- Uncles/aunts:
 - ◊ How did you feel acting out the role this time?
 - ◊ Was your relationship with your niece or nephew affected this time? How and why?
 - ◊ Were you willing to compromise?
- Ask full group:
 - ◊ What different techniques were used that helped the situation?
 - ◊ Were you both able to find a solution? Why? If so, were you both satisfied with this solution?
 - ◊ What kinds of solutions did you come up with?
 - ◊ How is negotiation helpful as a peer educator?
 - ◊ How is negotiation helpful in teaching your family and friends about healthy lifestyles?

Third briefing

16. Now explain that they will switch roles.
 - The person who previously played the niece or nephew will now play a Red Cross Red Crescent youth volunteer working with the healthy lifestyle programme. Your goal is to work with your peers in the community to inform them about healthy lifestyles and what they need to do to be and stay healthy.
 - The person who previously played the uncle or aunt will now play a youth in the community who does not know what a healthy lifestyle is. This youth eats too much fast food, drinks fizzy drinks instead of water and spends up to nine hours on the couch every day playing video games. When s/he can, s/he steals cigarettes from his/her father and smokes them when no one is at home. When s/he was caught drinking behind the school last week, his/her mother told him/her s/he must start attending the healthy lifestyle programme or s/he would lose his/her television privileges. His/her mother said she would check with the Red Cross Red Crescent youth volunteer every week to see if s/he is doing what s/he is supposed to do. Your goal is to keep your television privileges at home and keep your mother happy.

Third pairs negotiation

17. Once all participants can clearly visualize the situation, their roles and goals, invite the Red Cross Red Crescent youth volunteer to start a conversation with their partner community youth and engage in the role play whenever they feel ready to do so. In their pairs, they will now have a negotiation and try to reach a solution.
18. Allow up to ten minutes, then stop the negotiation processes.

Third process mapping

19. Ask all participants to sit on the side of the tape / line on the ground (standoff, compromise or consensus) which they think corresponds to the current situation they have just role-played.

Third plenary

20. Hold a short plenary:
 - Unhealthy youth:
 - ◊ How did you feel about your character? How did you feel acting out this role?
 - ◊ Did you get what you wanted?
 - ◊ Were you willing to compromise?
 - Red Cross Red Crescent youth volunteer:
 - ◊ How did you feel about your character? How did you feel acting out this role?
 - ◊ Did you get what you wanted?
 - ◊ Were you willing to compromise?
 - Ask full group:
 - ◊ What different techniques were used that helped the situation?
 - ◊ What kind of compromises did you come to?
 - ◊ What was the most difficult part of the negotiation?
 - ◊ How can we help others to live a healthier lifestyle?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

Developing the skill of collaborative negotiation helps us to identify and use appropriate methods and techniques to encourage dialogue and mutual understanding between people and create a conducive environment for building and reaching solutions that work for everybody. This allows us to be better equipped to inspire and influence people to make healthy choices and positively change their mindset, attitude and behaviour regarding healthy lifestyles choices.

Source: Adapted from YABC lesson: The dream come true

To better understand the notion of collaborative negotiation and its key ingredients, and be better prepared to facilitate this activity, please read the related YABC concept paper.

Effective communication skills

Time required

60 minutes

Audiences

- Youth peer educators
- Youth

Overview

Learners take part in three different types of communication activities to understand different ways of communicating with others and making themselves understood appropriately. These activities are fun, engaging and instructive.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Use appropriate techniques to positively influence peers.

Materials

- Make one copy of both communication sheets for every participant.
- Two images

Preparation

- Print one copy of communication sheets 1 and 2 for every two participants
- Make one copy of the two images for every two participants – do NOT print back-to-back

Process

Introduce the learning objectives and a brief overview of the activities in this session. Explain that they will engage in three different activities to understand the need for effective communication skills.

Activity 1

1. Clear chairs and tables from the middle of the room.
2. Explain that the next activity requires silence, i.e. no talking, but they can use hand signs to make themselves understood.
3. Explain that down the middle of the room, you would like everyone to get into one line from one wall to the other, BUT – without talking – you must all line up in the order of your birthdays. On one end of the room, anyone born on January 1 should be standing here and other January birthdays should be behind them – in order. As the line builds away from the wall towards the wall opposite, the birthdays should be progressing towards December 31 – where anyone with that birthday should be standing.
4. Ask to make sure they understand the directions, remind them to be silent and tell them to begin.
5. After they have formed the line, start at the January end and ask each person to state their birthday as you go down the line.
6. Ask everyone to take a seat and discuss:
 - How did you all do?
 - How did you communicate?
 - Who were your most effective communicators? Why were they so effective?

Activity 2

1. Everyone should find a partner. Each pair should bring one pencil and two pieces of paper with them as well as a book or hard cover where they can place their paper for writing.
2. Instruct the pairs to stand back-to-back with their partner in a straight line across the front of the room. They may not turn their heads or look behind them during the activity.
3. Tell the person facing away from you to take the pencil and paper. Explain that you will give the person in each pair facing you a picture.
4. Explain that the person with the picture must describe the picture in words to their partner. Their partner with the paper and pencil will draw the image that is being described on their blank paper using the description from their partner. Both people in the pair can ask the other questions but they may not look at each other's sheet until the facilitator calls time. Give them five minutes.

Lesson plan guidance

Effective communication skills

5. After five minutes, ask the pairs to turn around and share their pictures.
6. Once they have shared, ask them to switch places. The person who was drawing will now get a different picture while their partner must draw based on their description.

Activity 3

1. Explain that while there are many ways to communicate, it is important, especially as a peer educator, to use the most effective ways of communication and to ensure that communication remains a two-way process.
2. Explain that they will now work with a partner to learn about effective communication they learn about effective communication points.
3. Ask every participant to find a different partner. Instruct them to sit facing each other.
4. Give each pair a communication skills 1 sheet **and** a communication skills 2 sheet. Tell each person to take one of the sheets but not to show it to their partner.
5. They will take turns reading their sheet to their partner BUT the person reading must place their hands behind their back while the person listening must cover their eyes. They have one minute. Ask each pair to choose who will go first and start a timer.
6. After one minute, call SWITCH! Now the person who was listening reads their sheet with their hands behind their back while their partner covers their eyes.

Plenary

1. Ask the group:
 - How did that feel?
 - Did it feel comfortable or uncomfortable to tell the story to your partner? Why?
 - Did it feel comfortable or uncomfortable to hear the story with your eyes closed? Why?
 - Have you ever tried to communicate with people, but been ineffective? What happened? How was that?
 - What did you remember that your partner taught/read to you? Why did you have a hard time delivering the information to your partner?
 - Why did you have a hard time learning the information?

2. Now tell them that they will repeat the exercise, but this time they have no restrictions when telling or receiving. Ask them to start.
3. After one minute, tell them to switch in their pairs.
4. After another minute, ask the group:
 - Was that different? How? Why?
 - What content did you remember teaching your partner?
 - What content did you learn from your partner?
 - Was it easier to have no restrictions? Why?
 - How does this affect how you will communicate when you share about healthy lifestyles?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

Effective communication relies on many aspects including what we say, how we say it, our body language and whether we use physical contact. All these different elements contribute to a person's effectiveness when communicating and yet still vary from one person to another depending on their personality and comfort level when talking to others.

Environmental scan

Time required

75 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

This lesson provides youth an opportunity to conduct a physical environmental scan to identify obstacles and opportunities for healthy lifestyles. The environmental scan involves mapping health hazards in their immediate surroundings to develop a plan to reduce the things that make behaviour change more difficult. The scan also maps opportunities for healthier lifestyles.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Assess health behaviours and determine recommended actions.
- Explain the importance of supportive environments for healthy living.

Materials

For every five participants – one healthy life game packet which includes:

- 17 environmental scan cards per pair
- Markers

Preparation

- Open the environmental scan PowerPoint and print all pages. You may print as full-page cards or any appropriate size. Print one set of cards for every two participants. You may print back-to-back.
- Timer

Process

1. Introduce the learning objectives and give a brief overview of the activities in this session.
2. Explain that to make healthier choices, it is helpful to study closely the physical environment(s) where children and youth live, work and play. When we talk about environments here, we are not referring to pollution, but rather the settings where we work, play or live.
3. An environmental scan helps us to identify both healthy and unhealthy influences in our physical environment. This might include recording the number of advertisements for unhealthy foods and products, identifying if there are healthy food options available, how many unhealthy food options there are, whether there are places where children can exercise, etc.

The results of the scan help children and youth better understand how their environment helps or hurts their efforts to be healthier.

Explain that they will conduct a mini environmental scan of the area where they are meeting right now.

4. Tell the group that where they are sitting right now is the centre of their scan. They will work in teams only within 200 yards/metres or two city blocks of where they are right now.
5. They will receive a packet of environmental scan indicators and, working with a partner, they will identify the availability, accessibility and affordability of healthy and unhealthy food options, opportunities for physical activity etc. in the immediate area. They can also write notes or take a picture (if they have a camera) of what they find in their scan.

By mapping food stores, kiosks, restaurants, recreational facilities, street vendors, etc. within walking distance from the centre of the perimeter, teams and children and youth can see how easy or difficult it is for children and youth in that area to:

- Be exposed to advertisements for unhealthy foods and products
- Access unhealthy foods or products
- Engage in physical activity safely
- Engage in positive behaviours safely

After 30 minutes, everyone will return and share/compare their information. Using the results of the scan, the group will brainstorm how they can make their immediate environment healthier.

Lesson plan guidance

Environmental scan

6. Ask participants to find a partner.
7. Distribute 17 cards and markers to each pair.
8. Explain that they should stay within 200 metres of where they are standing now. Point out objects that are approximately 200 metres away to help them gauge the distance. You will call them back after 30 minutes.
9. After 30 minutes, call the groups back. Holding up each card, ask them what they found in relation to the question on the card. Write general observations on a flipchart or blackboard.
For each item they found, discuss:
 - How does this affect children and youth in this area?
 - Did you notice this before today?
 - What can we as a group do to promote a healthier setting?
 - After going through each card, ask how they feel? Empowered? Frustrated? Ask:
 - What can we do with this information to live healthier lives?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

Environmental scans for healthy lifestyles can be conducted to determine how many healthy options or resources there are and how many unhealthy options or barriers there are in a community or school's physical environment. Environmental scans can also identify policies and laws that might be making it more difficult to make healthier changes, such as unhealthy food being cheaper while fruits and vegetables are more expensive. A scan helps draw attention to the things that may make behaviour change difficult or easy and identify quick fixes for healthier lifestyles for a community or school community.

Gender in healthy lifestyles

Time required

90 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

This role play activity – adapted from YABC – allows learners to explore the social and cultural rules and norms regarding gender and lifestyle choices in their culture or region. Youth learn that these rules and norms often have a disproportionate effect on women and girls regarding healthy lifestyle and choices.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Use appropriate techniques to positively influence peers.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Assess health behaviours and determine recommended actions.
- Explain the importance of supportive environments for healthy living.

Materials

- Scenarios of daily-life situations sheet
- Dress up items (optional)

Preparation

- Print two copies of the scenarios sheet. Cut into strips.

Lesson plan guidance

Gender in healthy lifestyles

Process

1. Introduce the learning objectives and a brief overview of the activities in this session.
2. Explain that they will have experience today of doing some role playing around healthy and unhealthy behaviours.
3. Ask the participants to count off by fours. Ask the ones and the threes to join Facilitator 1 in the hallway or a separate area where the rest of the participants cannot hear or see them. Facilitator 2 will lead the twos and fours in a few simple exercises like sit-ups, squats or push-ups.
4. Facilitator 1 will ask the ones to pair up with a three for a role play event. Each pair will then be given a different scenario within a typical household to act out in front of the full group. Specify to men/boys that they should “act like men/boys in the household” and to women/girls that they should “act like women/girls in the household”. Allow them ten minutes preparation time.
5. When the ones and threes are ready, Facilitator 2 will explain to the twos and fours that they will watch the daily-life situation that a one and three pair will act out. The twos and fours will carefully observe the characters’ reactions. Once the role-players are ready, ask the ones and threes to come in and start performing their respective scenes.
6. When the performances are finished, invite participants to come together in order to reflect on this experience. You may use the following questions to guide the group discussions.

Ask the actors:

- How did you feel acting like someone of your own sex?

Ask the observers:

- What did you think of the performed scenes?

Ask all:

- Which characters’ reactions did the scenarios generate? Why? Think of whether or not the situations presented in the role plays reflect mindsets, attitudes, behaviours and roles of men/boys and women/girls in real life, etc.
7. Now, say that they will allow the other groups to also take part in a role play. Facilitator 1 will ask all the twos and fours to come out to the hallway. Facilitator 2 will lead the ones and threes in a few simple exercises like jumping jacks, squats or push-ups.

8. Facilitator 1 will explain that the twos and fours will be given the same roles as the pairs they just watched, but this time, they will switch roles and now they will focus on typical activities and routines of the different genders. An added twist is that men/boys have to “act like women/girls” and women/girls have to “act like men/boys”. They should act like the other gender doing what they feel are typical activities and routines for the other gender. They should all remain as serious as possible during their performance. They have ten minutes to prepare.
9. Once the role-players are ready, ask them to come back in and start performing their respective scenes.
10. When the role-plays finish, invite participants to reflect on the experience. You may use the questions below to guide the group discussion.

Ask actors:

- How did you feel acting as someone of the opposite sex?
- What did you think about when you were asked to “act like men/women”?

Ask observers:

- What did you think of the performed scenes the second time?

Ask all:

- Which characters’ reactions did the scenarios generate this time? Why? Think of whether or not the situations presented in the role plays reflect mindsets, attitudes, behaviours and roles of men/boys and women/girls in real life, etc.
- How can you relate this activity to real-life situations that you may have experienced or witnessed in your community? Think of:
 - ◇ the rules regarding the role of men and women, boys and girls, their origins and the way they influence mindsets, attitudes, behaviours and therefore relationships between the two sexes
 - ◇ the rules regarding permissions, lifestyle choices, and free choice given to each gender and how this affects the ability to make healthy choices and influence others to do the same
 - ◇ the consequences of these “norms” on the health of the concerned individuals (e.g. how putting men and women, boys and girls, in “gender boxes” may limit their lives and those of the people around them; what happens to human beings who try to live “outside these boxes” i.e. what people think about them, how they are treated, etc.)
 - ◇ How can we as agents of behavioural change to reduce NCD risk further engage in tackling societal expectations, stereotypes, prejudices and rules related to gender and therefore promote healthy lifestyle choices in our community?

Lesson plan guidance

Gender in healthy lifestyles

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

“Women and men have different levels of exposure and vulnerability to NCD risk factors. Women are significantly more likely to be obese than men. Women’s higher rate of obesity leads to their increased vulnerability to NCDs, particularly diabetes. Gender norms can predict current and future NCD risk. Social customs related to physical mobility may reduce women’s opportunities for activity, reflected in disparities between men and women in physical activity levels. Additionally, many societies view tobacco smoking as a desired masculine norm.”

Source: www.paho.org/hq/index.php?option=com_docman&task=doc_view&gid=17118&Itemid=

Scenarios of daily life situations

You are thinking about what you would like for dinner. You know you should eat something healthy but...

You are at a friends' house with a group of friends. Someone that you think is cute lights up a cigarette and shakes the pack at you, telling you to take one....

Your partner decides to start jogging three times a week and asks you to join.

Your father was just diagnosed with Type 2 diabetes and recently lost his foot to the disease. He is on a strict no-sugar diet. You come home from a fast food restaurant carrying a monster size soda and he demands that you give it to him...

You're watching TV and you have to cook and clean the house for your friends that are coming for dinner...

Today is Saturday, it's market day and you need to buy food for the family...

Your partner is participating in a sports competition so you decide to go and support the team...

You are at a party with your brother and they are serving alcohol. You are under age and have not yet tried alcohol....



Debate on healthy eating

Time required

90 minutes

Audiences

- Youth peer educators
- Youth

Overview

This activity teaches youth using debate, a behaviour-based methodology. Learners are expected to formally debate for a stance they do not necessarily believe themselves – to allow them to understand why people make the unhealthy choices that they do.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Explain the importance of supportive environments for healthy living.

Materials

- Five signs:
 - ◇ Poster A – Pro (for value line-up)
 - ◇ Poster B – Con (for value line-up)
 - ◇ Poster C – Debate statement
 - ◇ Poster D – Yes (for team debating for healthy eating)
 - ◇ Poster E – No (for team debating against healthy eating)
- Healthy eating debate guidelines poster
- Tape
- Large countdown timer which all can see. You may also use an online stopwatch (www.online-stopwatch.com/countdown-timer/) from the computer through an LCD projector.

Preparation

- Print out one copy of posters A to E.
- Make one copy of the Healthy eating debate guidelines poster.
- Tape the Pro and Con posters on the wall at different ends of the room.
- Clear the chairs and desks from the centre of the room.

Process

1. Introduce the learning objectives for the session. Give a very brief overview of the activities in this session and how the session will help them be better able to reach the objectives.
2. Explain to the group that you want their opinion about whether they do or do not eat healthy foods and drinks.
3. Point to signs posted on the walls.
4. State that the Pro poster is for people who **do** eat and **want always to eat healthy foods** and only have the occasional "junk food".
5. State that the Con poster is for people who **do not** eat or **do not want always to eat healthy foods** and only have the occasional "junk food".

If people are undecided, they may choose the side that suits them most.

6. Explain that you would like them to determine their position and form a value line between the two signs. They need to determine how they feel about the question and put themselves in one straight line with the person believing the strongest that they **must** eat and drink only healthy foods being at the far end under the Pro sign.

The person who believes the strongest that they will **not** eat and drink only healthy foods will be at the far end under the Con sign.

They all must talk and arrange themselves in order between Pro- Yes, I will **eat and drink only healthy foods** every day to Con- No, I will **not eat and drink only healthy foods every day**.

7. Ask them to get up and arrange themselves appropriately. As they are forming the value line, randomly question different people on the subject as well as the people around them to ensure they are in the correct position.
8. Once the line is formed, split the line in half and ask the half on the Pro side to sit on one side of the room. Ask the half on the Con side to sit on the other side of the room.
9. Explain that they will prepare and have a debate on the subject of healthy eating and drinking. Hold up Poster C and say that the statement they will debate is:

Everyone should eat and drink only healthy foods and drinks every day with only the occasional treat or junk food.

10. Tape the debate statement at the front of the room. Remove the PRO and CON posters.

Lesson plan guidance

Debate on healthy eating

11. Explain that you want to mix the teams up and make them think.

For the debate, the Con team will debate FOR the daily eating and drinking of only healthy foods. Tape Poster D – YES! near where the group is sitting.

The Pro team will debate AGAINST the daily eating and drinking of only healthy foods. Tape Poster E – NO! near where the group is sitting.

12. Explain that in their teams, they will have 15 minutes to prepare a set of arguments.
13. Each team will choose two representatives to debate with the two representatives from the other team. These two representatives will remain the same throughout the debate.
14. Point out the posted list of debate rules and go through each.
15. When everyone is clear on the rules and the procedure, ask if there are any questions.

Preparation

16. When everyone is ready, let teams get to work.
17. After 15 minutes, bring four chairs to the front of the room.
18. Ask for the two chosen representatives from each of the teams to come forward and sit. They may bring a pencil, blank paper and their notes with them.
19. Review the procedure and time for each round as listed on the Healthy eating debate guidelines poster. Answer any remaining questions.

Present points

As teams are about to begin, the trainer should keep track of statements presented as facts by the debaters which may need to be verified later and discussed with the group to avoid confusion. For example, a debater may claim that hamburgers are good for you. The trainer should keep track of statements like this and address these statements in a discussion after the debate.

20. Signal the Pro team to begin. Set the timer for three minutes.
21. After three minutes, tell them to stop.
22. Immediately, signal the Con team to begin. Start the timer again for three minutes.
23. After three minutes, tell them to stop.

Team discussion and preparation

24. Each team now has 1½ minutes to discuss among themselves and prepare for the next round to give a critique of the other team's points. Set the timer for 1½ minutes.

Critique points

25. After 1½ minutes, signal the Pro team to give a critique of the points made by the Con team in the last round. Start the timer for three minutes.
26. After three minutes, tell them to stop.
27. Immediately signal the Com team to critique the points made by the Pro team in the last round. Start the timer for three minutes.
28. After three minutes, tell them to stop.

Team discussion and preparation

29. Each team now has 1½ minutes to discuss amongst themselves and prepare for the next round where they will respond to the critique from the other team. Set the timer for 1½ minutes.

Responses to critique

30. After 1½ minutes, signal the Pro team to respond to the Con team's critique in the last round. Start the timer for three minutes.
31. After three minutes, tell them to stop.
32. Immediately signal the Con team to respond to the Pro team's critique from the last round. Start the timer for three minutes.

Team discussion and preparation

33. After three minutes, tell them to stop. Each team now has 1½ minutes to prepare a final argument.

Final arguments

34. After 1½ minutes, signal the Pro team to make their final argument. Set the timer for one minute.
35. After one minute, tell them to stop.
36. Immediately signal the Con team to make their final argument. Set the timer for one minute.

Lesson plan guidance

Debate on healthy eating

Discussion

- Have a full group discussion to help the group talk through what they are feeling after the debate. There will likely be some confusion, some frustration and lots of questions. The debate can become very emotional so it is critical to discuss with the group afterwards. Questions can include:
 - ◇ Do you feel differently now about the eating and drinking of healthy foods and drinks than you did before?
 - ◇ Explain how you feel differently.
 - ◇ What did you hear that was new to you?
 - ◇ You have probably heard many of these things before, but did anything change your mind from the debate?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

Teachers/peer educators often use the debate to effectively increase student involvement and participation during classes. When a teacher uses the debate, students gather supporting evidence, engage in collaborative learning, delegate tasks, improve communication skills, are able to see another person's point of view and rationale for doing what they do, and develop leadership and team skills. An excellent behaviour-based method.

Some helpful e-courses about using debate with youth can be found at: idebate.org/training/online

Healthy lifestyle game show!

Time required

60 minutes

Audiences

- Youth peer educators
- Youth
- Adults

Overview

Based on a television general knowledge game show, this PowerPoint deck allows the group to play a healthy lifestyles version of the show. Fun!

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Assess health behaviours and determine recommended actions.
- Explain the importance of supportive environments for healthy living.

Materials

- Healthy lifestyle game show PowerPoint
- LCD projector
- Flipchart and marker to record scores

Lesson plan guidance

Healthy lifestyle game show!

Process

1. Introduce the learning objectives and give a brief overview of the activities in this session.
2. Explain that they will be in groups of five participants each. Each participant in the group will take turns answering the question for their group.
3. There will be a game board projected on the wall. The answering participant from each group will stand at the front with you. When they finish, another person from their group comes to the front.
4. Appoint a scorekeeper.
5. Show the first screen of the Healthy lifestyle game show PowerPoint. Briefly discuss how the game works.
 - When a category and point value are chosen, the question is displayed.
 - You will read the question.
 - The first participant that does two push-ups will be called on to give an answer.
 - You will click the Next button to reveal the answer. You are the final judge as to whether a person answered correctly.
 - If they give a wrong answer, the next person to have completed the jumping jacks may answer, and so on until the correct answer is given.
 - The scorekeeper should keep score of the points that each team earns.
 - When you click the point value amount at the bottom of the answer screen, the full game board will show up again. The questions that have already been asked will appear in yellow.
 - The person who answers a question correctly may then choose the next question to be asked. The questions do not have to be asked in order.
 - If you click on a button and the Double healthy lifestyle game show button appears, the person who selected that category gets to decide how much they would wish to wager. They can only wager what the scorekeeper shows as "earned".
 - The game continues until all questions have been asked and you reach Final healthy lifestyle game show.
 - In Final healthy lifestyle game show, the participants from each group must decide how much of their group's "winnings" they would like to wager. The person with the correct answer and whose group "wins" the most money wins the game.

6. Start the game by choosing a category and price value. When you click, you will read the question out loud. The first person from the teams to complete two jumping jacks will be selected to answer the question. After Final healthy lifestyle game show has been completed and a winner has been established, ask everyone to come and sit in plenary. Some questions to include are:
 - ◇ How did you do?
 - ◇ Did you learn anything new?
 - ◇ What was the most difficult part of the game?
 - ◇ When you did not know an answer, did you discuss it with your teammates?
 - ◇ Which group provided for the most supportive environment for their teammates? How?

Wrap-up

Review the session objectives. Answer any remaining questions.

No projector?

No problem. If you do not have access to a projector, print out the slides of the PowerPoint BACK-TO-BACK. Play the game the same way but hold up the question card to participants – you can see the answer on the reverse. You will need to organize the cards by categories to keep track of questions that have been asked already.

Healthy lifestyle relay

Time required

45 – 60 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for younger children

Overview

Learners will be placed into teams and will compete for points by answering knowledge questions, performing healthy lifestyles skills/activities and working together to complete healthy lifestyles puzzles.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Assess health behaviours and determine recommended actions.
- Identify the importance of consistently engaging in healthy lifestyles activities.

Materials

- Knowledge challenge sheet
- Activity challenge sheet
- Puzzle board sheets
- Point cards
- Pitcher of water and three cups
- Three bananas

Preparation

- Print one copy of the puzzle board sheets for every five participants. Cut along the outer and inner lines of each healthy lifestyle poster to make a puzzle. Place puzzle pieces from each puzzle into individual envelopes or Ziploc bags. You may choose to laminate these cards before cutting to preserve for future sessions.
- Print one copy of the knowledge challenge sheet.
- Print one copy of the activity challenge sheet.
- Print four copies of point cards and cut out.

Process

1. Introduce the learning objectives for the session and give a brief overview of the activities in this session.
2. Divide the class into groups of five participants per group.
3. Ask for two volunteers to help you with assigning points throughout the relay. You may also switch people in and out of these point-giver roles throughout the activity so that everyone gets a chance to play. To do this, explain the process of assigning points to the full group so that people who are asked to be a points-giver know what they should do when they are called upon.
4. Ask each group to get into individual group lines. The person at the beginning of each line should face you with their group members behind them.
5. Explain that they will all take part in a relay of knowledge, skill and activity capacities to test their healthy lifestyle expertise. The persons at the front of each line will all be asked either a knowledge question or will be expected to perform a skill or an activity. After you ask the question or announce the skill, the first person who answers a question or completes the skill activity correctly wins one point card and everyone at the front of the line now goes to the end of their group line.
6. You will continue with the knowledge question or skill or activity with the next person, all of whom are now at the front of the line.. Again, the first to complete successfully wins a point. All the people at the front then go to the back of the line. Continue until all knowledge questions, skills and activities have been completed.
7. Each group must keep track of their point cards.
8. When all knowledge questions, skills and activities have been completed, the group with the most points wins
9. The group which completes all the challenges and completes the puzzle correctly in the fastest time is the winner
10. Answer any questions about the game, then ask participants to get into their groups and begin play
11. After the game finishes, ask the groups to get together for a discussion. Questions may include:
 - How did the game make you feel?
 - What was easy in the game? Was the game difficult?
 - What new knowledge did you learn?
 - How can you use this game with your friends?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

“Noncommunicable diseases (NCDs), primarily cardiovascular diseases, cancers, chronic respiratory diseases and diabetes, are responsible for 63 per cent of all deaths worldwide (36 million out of 57 million global deaths).”

NCDs are especially important for young people, now and in the future. Two thirds of premature deaths in adults are associated with childhood conditions and behaviours, and behaviour associated with NCD risk factors is common in young people: over 150 million young people smoke; 81 per cent of adolescents do not get enough physical activity; 11.7 per cent of adolescents partake in heavy episodic drinking and 41 million children under five years old are overweight or obese. Apathy to change current behaviours will add to the current NCD burden, with severe consequences for future populations and their health systems. Today’s youth are tomorrow’s leaders and carers will bear the brunt of these costs, both financially and personally. Youth everywhere therefore have a vested interest in NCD prevention.

Sources:

http://www.who.int/features/factfiles/noncommunicable_diseases/en/

<http://www.who.int/global-coordination-mechanism/ncd-themes/ncd-and-youth/en/>

Activity challenges

1. Do 15 jumping jacks or sit-ups.
2. Hold a plank pose for 10 seconds. Get an additional point for each additional 10 seconds.
3. Do 10 crunches.
4. Do five push-ups – keep your back straight!
5. Keep your legs straight and touch your toes.
6. Drink two glasses of water.
7. Sing the song “Head shoulders knees and toes” or a local variation five times while you reach and touch each body part. Speed up your singing each time.
8. Hold a banana with one hand and peel the fruit with only one finger on your other hand. Eat the banana!
9. Do 10 deep squats.
10. Name five different kinds of vegetables or fruits that grow where you live.

Knowledge challenge

1. Name one noncommunicable disease and name at least two parts of the body that are negatively affected by the NCD.
2. Name two risk factors of NCDs.
3. How many daily servings of fruits and vegetables are recommended for NCD prevention?
4. Which NCDs can be caused by smoking?
5. Which NCDs can be caused by drinking too much alcohol?
6. Which NCDs can be caused by physical inactivity?
7. Which NCDs can be caused by unhealthy diet?
8. Give one good reason why it is important for YOU to have a healthy lifestyle and another reason why your family should lead a healthy lifestyle.
9. Name one of your favourite foods and tell how you can make that food a healthier option.
10. Name one example of physical activity that you can do on your way to school.
11. True or False - NCDs are highly infectious.
12. True or False. Exercise is the only way to lose weight regardless of what you eat.
13. True or False. Exercise can help to control high blood pressure.
14. True or False. It is impossible to make my traditional food healthier.
15. True or False. Shisha and water pipes are 200 per cent deadlier than a cigarette.

Lesson plan guidance

Name that activity

Name that activity

Time required

30 – 40 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for younger children

Overview

Learners will identify and discuss locally available resources for preventing NCDs to which youth have access. For each resource, they will discuss activities they can do to live healthier lives.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Identify the four risk factors for NCDs.
- Explain the importance of supportive environments for healthy living.

Materials

- Metacards
- Four flipchart sheets
- 10 markers
- Four sets of pictures representing locally available resources
- Envelopes
- Tape

Preparation

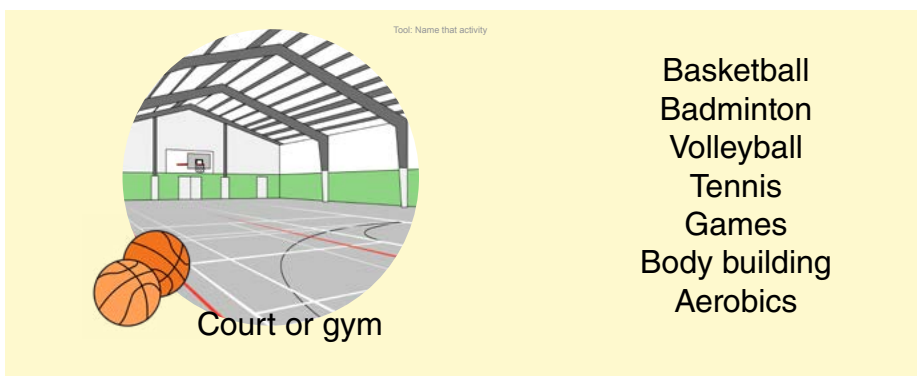
- Print four copies of the metacards. For each set, cut into individual cards and place in an envelope.
- Obtain four pieces of flipchart paper.

Process

1. Introduce the learning objectives and give a brief overview of the activities in this session.
2. Divide the participants into four groups.
3. Distribute 16 metacards, one piece of flipchart paper, one long piece of tape and several markers to each group.
4. Tell them to look at the cards. They should tape each card on to the flipchart paper and leave space around the card to write in activities associated with each resource.

Once they have taped all the cards onto the paper, explain to them that the pictures are locally available resources which will help enable them to do activities that may prevent NCDs. If they are unsure what a picture is meant to represent, they should take their best guess.

In their groups, they will brainstorm **for each resource shown** - all the activities that they can do with that resource to prevent NCDs. They should write their ideas down on the flipchart paper around the edges of the specific resource. Here is an example:



They will be given 15 minutes to work on it.

5. After 15 minutes, call the groups to attention.
6. Ask each group to post their flipchart sheets. Point to each resource image and take a few minutes to hold a plenary about EACH resource listed:
 - a. What is this resource?
 - b. How can this resource be used to prevent NCDs?
 - c. Do you know where to find this resource where you live, work or play? Where?
 - d. What activities did your groups come up with to use this specific resource?
 - e. How likely are you to use this resource?
 - f. How can you support others to use this resource?

Wrap-up

Review the session objectives. Answer any remaining questions.

Name that risk factor

Time required

45 – 60 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for younger children

Overview

This activity teaches youth about the four risk factors that contribute to noncommunicable diseases and helps them understand which are preventable and which are non-preventable risks. This information is then put into play through a relay where participants are placed in teams and are given risk factor prompts which they must identify as preventable or non-preventable. For each preventable risk factor, the group identifies low-cost and easy access activities to combat the risk.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Identify the four risk factors for NCDs.
- Assess health behaviours and determine recommended actions.

Materials

- Two flipchart sheets
- Markers
- One copy of the “risk factor” cards for both Team 1 and Team 2
- Tape
- One stack of sticky notes

Preparation

- Print out two copies of the risk factor cards.
- Cut off 36 small pieces of tape and attach them to the edge of the flipchart stand for easy access.

Process

1. Introduce yourself, the activity and the objective of the activity.
2. Put up two flipcharts where the participants can clearly see and reach them. On one flipchart, write PREVENTABLE RISK FACTORS, and on the other, write NON-PREVENTABLE RISK FACTORS.
3. Explain what a risk factor is and how doing an NCD risky behaviour puts you at greater risk of having an NCD. Use the four "Effects of..." posters to teach or remind the group what these risk factors can do to their NCD risk.

Explain the difference between preventable and non-preventable risk factors.

4. Show the participants the stack of risk factor cards. Explain that each card lists one risk factor that contributes to NCDs.

Let's play!

5. Divide the group into two teams. Ask the two teams to line up facing the two flipcharts.
6. Explain that the first person of each team will receive a stack of cards – each listing an NCD factor. When you call "Go", the first person should take the top card and hand the rest of the cards to the person behind them. The first person should then race to the flipcharts and tape their card to the correct flipchart sheet, then join the back of their line. Once they have returned to the team, the next person takes the top card, hands the whole stack to the person behind them, and races to the front to tape their card to the correct sheet. Play should continue until all cards have been placed.
7. Ask the two teams to sit down and then request one person to come up and be the scorekeeper. Whichever team finished placing the cards first automatically receives one point which the scorekeeper should record.
8. Review in plenary how the cards were placed. Facilitate going through each risk factor and determine as a group if it was placed correctly. Each time that a team places the card correctly, the scorekeeper assigns one point. Team affiliation can be determined as one teams' cards are in black ink and the other team's cards are in blue ink.
9. If a card was placed incorrectly, ask the group to discuss where it should go and then place it correctly.
10. Now give each participant a blank sticky note. Explain that they will discuss as a group how they can stop doing the preventable risk factors using inexpensive or free activities like going for a walk to be physical or having an active school garden to grow fruits and vegetables.

Lesson plan guidance

Name that risk factor

11. For each risk factor and solution discussed, select one participant to write one of these solutions on a sticky note and come up to place the solution on the appropriate risk factor. Do not wait for them to write and post - continue to go through each risk factor assigning participants to write solutions to keep the activity moving.
12. After all risk factors have been discussed and the solution has been posted, have a discussion in plenary. Some suggested questions include:
 - Which category (preventable and non-preventable) has more risk factors?
 - Do people want to have cancer or diabetes or have a heart attack or have a lung disease? Why do you think then that people continue to practice bad behaviours if it puts them at risk for NCDs?
 - How can you prevent having an NCD?
 - How can you prevent your family from having an NCD?
 - What are the effects on your family if you have an NCD?

Wrap-up

Review the learning objectives for the session and discuss how the session helped them to better learn the content.

Background information

Modifiable behavioural risk factors

Tobacco use, physical inactivity, unhealthy diet and the harmful use of alcohol increase the risk of NCDs and are all preventable risk factors.

- Tobacco accounts for around 6 million deaths every year (including from the effects of exposure to second-hand smoke), and is projected to increase to 8 million by 2030.
- About 3.2 million deaths annually can be attributed to insufficient physical activity.
- More than half of the 3.3 million annual deaths from harmful drinking are from NCDs 1 .
- In 2010, 1.7 million annual deaths from cardiovascular causes have been attributed to excess salt/sodium intake.

Metabolic/physiological risk factors

These preventable behaviours lead to four key metabolic/physiological changes that increase the risk of NCDs: raised blood pressure, overweight/obesity, hyperglycaemia (high blood glucose levels) and hyperlipidaemia (high levels of fat in the blood).

In terms of attributable deaths, the leading metabolic risk factor globally is elevated blood pressure (to which 18 per cent of global deaths are attributed) (1) followed by overweight and obesity and raised blood glucose. Low- and middle-income countries are witnessing the fastest rise in overweight young children.

Non-preventable risk factors include age, gender, genetics or family history of NCDs.

Source: www.who.int/mediacentre/factsheets/fs355/en/

Peer pressure

Time required

45 minutes

Audiences

- Youth peer educators
- Youth

Overview

Learners are exposed to peer pressure and explore effective strategies both to use its power in favour of healthy lifestyles and to resist the temptation of engaging in risky behaviours.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Explain the importance of supportive environments for healthy living.

Materials

- Chocolates (you can substitute a soft drink, doughnuts or candy) – two per participant plus a few extra
- Flipchart sheet or blackboard with the following steps:
 - ◇ Take a deep breath.
 - ◇ Focus on finding the words you can say to the person pressuring you.
 - ◇ Think it through – will this get you the outcome you really want?
 - ◇ Think about what you could do instead of giving in to temptation.
 - ◇ If possible - walk away.
 - ◇ Congratulate yourself!! You did well.
 - ◇ If you were unable to resist – it's okay. But what would you do differently next time?

Preparation

- Pre-prepare a flipchart sheet or blackboard with the steps to resist peer pressure

Process

1. Introduce the session, explaining to participants that they will undergo an activity to help them self-assess their resilience and persuasiveness.
2. Divide participants into pairs and give each participant a piece of chocolate.
3. Ask the pairs to self-assign each person to be Person A or Person B. Then, take aside and brief **the As and Bs separately**:
 - The A participants should eat their chocolate in front of their partner and try to tempt and convince them to also eat their candy.
 - The B participants should resist eating their chocolate.
4. Once all the participants are clear with the instructions, invite the As to begin eating their chocolates and tempting their partner.
5. After a few minutes, stop and inform the Bs that the process is not yet complete and they should not eat their chocolate.
6. Hold a plenary to discuss the process so far. Questions can include:
 - Group A: How did this activity feel? Why? Was it easy or difficult to tempt and convince others to eat their candy? What techniques or strategies did you use to persuade them?
 - Group B: How did this activity feel? Why? Was it difficult or easy to refuse to eat the candy? What techniques or strategies did you use to resist the pressure?
7. Explain that there are ways to resist temptation or peer pressure, especially when you really do not want to do what someone else is trying to encourage you to do or when you do want to do something that others are encouraging you not to do. You can always take action to defend against peer pressure. Read the steps out loud. They are:
 - Take a deep breath.
 - Focus on finding the words you can say to the person pressuring you.
 - Think it through – will this get you the outcome you really want?
 - Think about what you could do instead of giving in to temptation.
 - If possible - walk away.
 - Congratulate yourself!! You did well.
8. Ask for two volunteers to demonstrate this process while you narrate the process steps.
9. When the volunteers are in front of you, give both of them another piece of chocolate. Now tell one of them to try to tempt the other to eat their chocolate. After 30 seconds of temptation, slowly read the steps to defend against peer pressure. Tell the person who is trying to resist temptation to follow the steps as you read them.

10. Ask the participants to get back into their original pairs.
11. Give a new chocolate to each A participant. B participants should still have their chocolates. If not, give them another and say out loud for all to hear, "If you were unable to resist – it's okay. But what would you do differently next time?" Wait for their answer.
12. Now B participants will eat both of their chocolates and try to tempt their partner. After 20 seconds, say "Remember your steps to resist! Practise resisting peer pressure, all As! You can do it."
13. After a few minutes, ask if they have suggestions for other strategies to resist temptation. On a separate flipchart, write ideas given by the participants. Explain that while they may follow the steps given on the first flipchart, any strategy that works for them is the best strategy, so feel free to experiment.
14. Now inform the As that the process is now complete and they can eat their chocolate if they like.
15. Hold a plenary to discuss the process. Questions can include:
 - **Bs:**
 - ◇ How did this activity feel? Why?
 - ◇ Was it easy or difficult to tempt and convince others to eat their candy?
 - ◇ Did you feel a little revenge against your partner because of what they may have done to tempt you in the first round? How did you work harder to tempt them?
 - ◇ If you had to ask for another chocolate because you ate yours before this round could begin, how did you feel? Embarrassed? Satisfied? How can we support people who give in to temptation but want to keep trying to resist peer pressure?
 - **As:**
 - ◇ How did this activity feel? Why?
 - ◇ Was it difficult or easy to refuse to eat the candy?
 - ◇ Did the steps to resist peer pressure help? How?
 - ◇ Was it easier to resist because you had already had a piece of chocolate? If so, how can we use timing to ensure that our behaviour change strategies are most effective?
 - **For both:**
 - ◇ How can we further engage in preventing risky behaviours or protecting both ourselves and others in potentially dangerous situations?
 - ◇ How can we NOT resist peer pressure when we are being pushed to take part **healthy activities** like physical activity or eating vegetables?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

As agents of behavioural change, we need to be aware of the pressure that our peers or community may put on us. We also need to understand how our personal decisions and actions may affect others.

Developing personal resilience will help us to take decisions and act from a principle of inner listening based on independent critical thinking, and therefore to resist and respond effectively to pressure in any situation, no matter how tempting it may be. As a result, we will be better equipped to inspire and influence a positive transformation of mindsets, attitudes and behaviours towards healthy lifestyles.

Source: YABC toolkit, Module 6: Intra and interpersonal skills - Temptation, which was inspired by materials developed by the Thai Red Cross. To better understand the notion of personal resilience, and be better prepared to facilitate this activity, please read the related YABC concept paper.

Building personal resilience

Time required

90 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

This adaptation of a YABC lesson allows learners to experience different methods and techniques to respond effectively to situations of peer pressure which encourage risky attitudes and behaviours.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Explain the importance of supportive environments for healthy living.

Materials

- A list of statements for each aspect of healthy lifestyles
- A flipchart or whiteboard
- Markers or chalk

Preparation

- Make one copy of the "Themes and related lists of statements" sheet.
- Prepare four flipchart sheets titled "Unhealthy diet", "Tobacco", "Alcohol" and "Physical inactivity" with the statements for each listed underneath.

Process

1. Introduce the session by explaining to participants that they will be exposed, through role- playing, to different risky situations.
2. Divide participants into four groups.
3. Separately provide each group with a specific theme (alcohol, tobacco, unhealthy diet or physical inactivity) and the corresponding list of statements for their theme.
4. Then locate each group in a corner or a separate area as far away from other groups as possible. Explain that each corner or marked area represents a particular health risk situation they may encounter.
5. In their group, they should prepare a short role-play lasting no more than two minutes using the list of messages provided. This role play is to present a situation of peer pressure related to the risky behaviour of their particular theme. They have 10 minutes to prepare.
6. Once all groups have developed a scenario of peer pressure corresponding to their topic using the statements, invite each group to perform the role-play twice in front of the others.
7. Explain to participants that the first performance is to set the scene and present the risky situation.
8. The second performance should be a repeat of the first performance. But in this second performance, you will ask several people from the audience to come up into the scene and intervene to positively influence the outcome. For every peer pressure message that is given, the selected person from the audience will be expected to counter that message with a technique or other message to resist the pressure and respond to the situation effectively.
9. After the group finishes their second performance, show the flipchart sheet for the theme that was just presented with the corresponding peer pressure statements. For each statement, prompt the audience to say what the response was to counter the peer pressure message. If none was given during the performance or if the response was inappropriate ask for other responses that could have been given.
10. As you discuss each theme, performance and responses to peer pressure messages, facilitate a discussion with the full group. Questions may include:
 - a. How did you feel about the scene? Has this ever happened to you?
 - b. How were the counter messages? Were they effective? Did they make sense? Would they work in a situation that happened to you or to a friend?
 - c. Can you see yourself using some of these counter messages?

11. Ask the group that performed to have a seat and move onto the next team following the same procedure. Continue until all four groups have performed twice and counter messages have been presented and discussed as a group.

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

Peer pressure is a powerful force that can make people do things they do not necessarily want to do, in an attempt to avoid “losing face” with people in their peer group. Peer pressure can also be used to positively persuade others to make healthier choices. By allowing youth to examine a situation using typical peer pressure statements and come up with effective and appropriate counter statements, one can better prepare youth to handle the next situation where they are pressured. Allow them to practise these lines and learn how to walk away when they are uncomfortable. This is a skill that needs to be practised and honed.

Source: YABC Lesson Plan “Let’s celebrate”.

To better understand the notion of personal resilience, and be better prepared to facilitate this activity, please read the related YABC concept paper.

Playing my role

Time required

90 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for younger children

Overview

Learners are placed into a role play group and assigned a specific character or role to play. Each role play group receives a different scenario to act out to the full group with a brief discussion afterwards. This lesson will help peer educators become aware of situations and experiences in their work.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Assess health behaviours and determine recommended actions.

Materials

- Scenario slips
- Role slips
- Name tags/sticky notes for each participant to identify their role

Preparation

- Make one copy of the character role slips sheet for every six participants in the full group.

Process

1. Introduce the learning objectives for the session and a brief overview of the activities which will help them reach the objectives.
2. Review the stages of behaviour change.
3. Ask the participants to form groups of six participants each. If you have extra participants, form one smaller group (but no less than three in the one smaller group; if fewer than three, re-distribute those few to the other groups).
4. Give each group a small stack of the six role play strips and name tags.
5. Let each member read and reflect on their character. They should make their name tag with their character name.
6. Randomly distribute one unique scenario per group.
7. Let each group discuss how they are going to act out their roles in relation to their given scenario.
8. Give them 10 minutes to plan.
9. Let the groups present their act for five minutes in front of the plenary. Each group has one minute to set up, five minutes to present. After each group presents, conduct a brief overview discussion, including:
 - How did the group handle the given scenario?
 - What were some of the challenges in this scenario? Who experienced the challenges? How did they manage these? What would you have done differently?
 - Have you ever been in a scenario like this? What did you do?
 - What were the strategies taken to change unhealthy behaviours?
 - What could have been done differently to effectively change unhealthy behaviours?
 - What stage/s of behaviour change was/were discussed?
 - What are the practical applications of this activity in real life?
 - Was the group able to present healthy lifestyle behaviours? What are they?
10. After all presentations, discuss/process the following using the discussion points below:
 - Have you already experienced scenarios similar to these at some point? What did you do?
 - What other scenarios can you share?
 - What was the most difficult part of being the peer educator in these scenarios?

Lesson plan guidance

Playing my role

Wrap-up

Review the achievement of the learning objectives.

Background information

Peer education is one of the most effective methods for bringing about lasting behaviour change, which can be challenging for many. Peer educators can rely on numerous strategies to handle the wide range of situations they may be confronted with in their work as behaviour change agents. The IFRC's Youth as Agents of Behavioural Change (YABC) initiative equips youth with strategies to manage peer interactions effectively.

Project: artwork

Time required

2 hours

Audiences

- Youth peer educators
- Youth

Overview

Youth learn the three elements that make behaviour change messages effective. They see examples and practise evaluating messages for effectiveness. Once they understand what is needed to make an effective message, they work together to create their own effective behaviour change messages for their youth/peers about healthy lifestyle. This activity demonstrates the power of the emotional/psychological dimension to trigger the behavioural change process.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four risk factors for NCDs.
- Identify how they can consistently engage in healthy lifestyle behaviours.
- Use appropriate techniques to positively influence peers.
- Explain the importance of supportive environments for healthy living.

Materials

- Assorted art materials. These can be anything you have handy such as recycled materials like scrap paper, paper, yarn, fabric, stamps, magazine cut-outs, plastic, raffia, foil, labels, lids, matchsticks, corks, natural items (bark, leaves, seeds, eggshells, seashells, twigs, etc.), buttons, and colouring materials like crayons, markers, ink, paint, chalk, etc.
- LCD projector. If you do not have one, print out the PowerPoint slides as handouts for use in the presentation
- Behaviour change messaging PowerPoint/handouts

Preparation

- Gather art materials for participants to use
- On a flipchart, write the three elements of effective messaging:
 - ◇ Simple
 - ◇ Call to action
 - ◇ Communicate a benefit (for long-term behaviour change)

Process

1. Introduce the learning objectives and give a brief overview of the activities in this session.
2. Refer to the flipchart and review the three elements that are necessary for developing an effective behaviour change message, poster or image.
 - Simple
 - Call to action
 - Communicate a benefit (for long-term behaviour change)
3. Show the PowerPoint presentation. As you open each health promotion image, show it for 30 seconds and ask the group to evaluate the message using the three criteria of effective messaging. Ask the group to help you evaluate whether each message is an effective long-term behaviour change message. Ask: If not – why not? How could they improve the message to make it effective?
4. Once they have a good idea of what effective messages look like, tell participants to get themselves into groups of four to five people.
5. Explain that each group will be tasked to create an effective message poster, flyer, brochure, collage, billboard, t-shirt design, etc. about healthy lifestyle. The artwork should relate an effective message to encourage others to live a healthy lifestyle. Each group's work should meet the elements of effective messaging.

Explain that they have 60 minutes to create a design and then they will present their work to the other groups for a critique of whether their message meets the three criteria.

6. After 60 minutes, ask each group in turn to present their work. Facilitate a critique of each group's work against the three elements of effective messaging. If a group failed to meet all three criteria, ask the full group to give suggestions on how they could modify their work to make it more effective.
7. When each group has presented and been evaluated, hold a plenary discussion. Questions may include:
 - Why is it important to be able to evaluate if a message is effective or not?
 - What can you do if you see an ineffective public message in your community? Why is it important?
 - How can you use this information to teach healthy lifestyle choices to your family and peers?
 - Take a photo or draw/write one effective and one ineffective message you see in your community. Bring them to group next session and present for discussion.

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

For community schools and groups, organizations, and at the national level, ensure that you prepare Information, Education and Communication (IEC) materials, billboards, and other promotional messages that are appropriate to your audience and address the barriers that keep them from doing healthy behaviours. In developing health messages, they should meet the following three criteria:

- Communicate a benefit
- Keep it simple
- A call to action

Source: IFRC Behaviour change framework, 2013

Someone else's shoes

Time required

45 minutes

Audiences

- Youth peer educators
- Youth

Overview

This adaptation of a YABC lesson allows learners to put themselves into the shoes of a specific character. Based on their character's identity and life situation, they physically respond to a series of questions and then compare their final position with that of others. This lesson allows young people to experience some of the challenges - including unequal access to opportunities, stigmatization and discrimination - that people may face due to their life situation and choices.

Learning objectives

At the completion of this topic, learners will be better able to:

- Explain the importance of supportive environments for healthy living.

Materials

- Pieces of scrap paper – 10 per participant
- Chalk, string or tape to mark a line on the floor
- Character description cards sheet
- List of possible questions sheet

Preparation

- Mark on the floor with chalk, string or tape, the same number of evenly spaced lines as there are questions to be read out to the group.
- Make one copy of the character description cards sheet for every 20 participants. Cut the cards into individual meta-cards.
- Make one copy of the list of possible questions sheet.

Process

1. Ask participants to stand side-by-side, behind the starting line at the back of the room facing you.
2. Hand out a different character description to each participant and tell them not to share it with anybody. If they have questions about their character, they must ask you privately. They can use their imaginations and develop their characters. They must, however, remain true to the description provided.
3. Give each participant four pieces of scrap paper and instruct them to ball each piece of paper up separately.
4. Explain to participants that you will read out a number of questions related to daily life to which they will have to react, based on their character's identity and situation.
5. Tell them that when each question is read out:
 - Those who do not have enough information on their character description to answer the question and those who think their answer to the question is "no" must stay where they are.
 - Those who think they can answer "yes" to the question should take a step forward (i.e. move on to the next line marked out with chalk, string or tape on the floor) and throw their paper balls at those who stayed in place and are now standing behind them.
6. Remind participants that they are to respond to the questions as they think their character would and not as they themselves would. Add that those who have not taken a step forward are not allowed to throw any paper balls.
7. When everyone is clear about the instructions, invite participants to close their eyes, clear their minds and relax. Tell them think about their character and to visualize that person, to try to see, hear and feel that person, to imagine their life and make it their own. Give participants a few minutes to put themselves into their character's shoes and to think about what their relationships to those around them would be like. After a few minutes, invite them to slowly open their eyes.
8. With everyone ready, start to read out the questions one at a time. Remind people of the rules for physically responding to each question.
9. Once all questions have been read out, tell participants to remain where they are and look around, memorizing their position in comparison to that of others.

Lesson plan guidance

Someone else's shoes

10. Invite everyone to sit down where they ended up at the end of the game and ask each participant in turn to read out their character description.
11. Once all characters have been introduced, end the game and invite all participants to sit together for a debriefing of this experience in plenary. Questions may include:
 - How did you feel about your character? Was their identity and situation much different from your own? Why or why not?
 - How did you feel when you took steps forward?
 - How did you feel when having to stay where you were when others were moving ahead?
 - How did you feel when throwing paper balls at those standing behind you?
 - How did you feel when others threw paper balls at you?
 - How did you feel about other people's reactions when you revealed the identity and situation of your character at the end of the game?
 - What did you notice at the end of the game when you looked around and compared your position to that of others?
 - How does this game motivate you to adopt and advocate for healthy lifestyles?
 - How can you make sure to recognize and provide support for those peers who are struggling but may not outwardly show it?

Wrap-up

Review the achievement of the learning objectives.

Background information

As agents of behavioural change, we need to be aware of and understand the stigmatization, marginalization and discrimination that many people face due to preconceived ideas, stereotypes and prejudices which limit their daily lives, such as unequal access to life's opportunities, unequal treatment, oppression, exclusion, violence, etc.

Developing intra-and interpersonal skills, such as empathy, critical thinking, dropping bias and non-judgement help us to reduce stigmatization, respect diversity and encourage the creation of a social environment where vulnerable groups will feel empowered, valued and included. As a result, we will be better equipped to inspire and influence a positive transformation of mindset, attitudes and behaviours towards equality and healthier choices in society.

Source: YABC Lesson Plan "Car park".

To better understand the notions of stigmatization, marginalisation and discrimination, and be better prepared to facilitate this activity, please read the related YABC concept paper.

Special effects

Time required

45 minutes

Audiences

- Youth peer educators
- Youth
- Can be adapted for adults

Overview

Learners work in groups to explore the rationale of why people make both unhealthy and healthy choices. Through exploring the bad and good effects of both types of behaviours, peer educators understand why people do what they do and are better able to come up with appropriate strategies and activities in response. The exercise also identifies barriers that keep people from continuing to make healthier choices.

Learning objectives

At the completion of this topic, learners will be better able to:

- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Assess health behaviours and determine recommended actions.
- Explain the importance of supportive environments for healthy living.

Materials

- Any makeshift divider
- Four sticky notepads - each one a unique colour
- Timer
- A4 sheets – each printed with a unique heading:
 - ◇ Good effects of healthy lifestyle to self, family and community
 - ◇ Bad effects of healthy lifestyle to self, family and community
 - ◇ Good effects of unhealthy lifestyle to self, family and community
 - ◇ Bad effects of unhealthy lifestyle to self, family and community

Preparation

- Print A4 tools
- Tape/post the four sheets at the front of the room

Process

1. Introduce the learning objectives and give a brief overview of the activities in this session.
2. Explain that the group will be divided into four groups. Each group will receive a pad of sticky notes in one specific colour.
3. Point out the four sheets with a unique prompt on each.
4. Explain each sheet. We often hear about the good effects of a healthy lifestyle and we consider the bad effects of an unhealthy lifestyle. But to better understand why people fail to lead a healthy lifestyle, it may be important to understand the perceived good effects of leading an unhealthy lifestyle and the supposed bad effects of leading a healthy lifestyle. By understanding why people do what they do or fail to do even though it is unhealthy, we are better able to provide support and a plan to help them make healthier choices. This activity allows the participants to consider different reasons why people engage in unhealthy behaviours or fail to engage in healthy behaviours. The activity also reinforces the good effects of a healthy lifestyle and the bad effects of an unhealthy lifestyle.
5. Explain that all groups will focus on the same prompt or sheet at one time. You will announce which sheet they will focus on and all groups have three minutes to brainstorm and write their ideas/effects to the assigned prompt on their sticky notes. Each group will have its own colour and one sticky note per submission will be expected. After a person in their group writes an idea or effect on a sticky note, a member of their group will run up and place their sticky note on the wall below the prompt. Each group member takes a turn in placing one sticky note on the wall. Once everyone has placed at least once, group members may then place another note and so on. Even if they are not eligible to place a note, they are encouraged to help the rest of their group brainstorm.

Explain that when three minutes are up, you will appoint a scorekeeper. Explain that you will go through all sticky notes for that prompt and read each one aloud. You ask the full group to give an “okay” gesture or “thumbs-up” if the contribution is a valid or appropriate response or give a “thumbs down” if the contribution is not valid or appropriate to the prompt. If it is voted valid, you will see if the same response/idea has also been submitted by another team. If no other team has submitted that idea/response, the submitting team gets a point. A point is only awarded to a team for an idea that is valid and has not been submitted by any other team. The goal is for your team to identify as many unique ideas as possible to a specific prompt.

Answer any questions they may have.

6. Divide the class into four groups. Give each group a pad of sticky notes in a colour that is unique to just their group.

7. Turn to the prompt/sheet that says, "Good effects of a healthy lifestyle to self, family and community". Start the timer for three minutes and tell the groups they may begin.
8. During the three minutes, monitor to make sure all participants are taking part in brainstorming and posting notes.
9. After three minutes, call "STOP – pens down!"
10. Go through the sticky notes, reading each one and discussing if it is a valid contribution. If the room gives the idea an "okay" sign or a "thumbs up" that it is valid and if no other group has submitted that response, tell the scorekeeper to give the contributing team one point. Go through each of the responses similarly - reading aloud, asking if it is valid, seeing if another team has made the same contribution and awarding points, as appropriate.
11. Now move to another prompt and continue the process.
12. Proceed through each of the four prompts using the process above.
13. Once all four prompts have been addressed, determine the winning group.
14. Bring all groups together for discussion. Some questions may include:
 - What kinds of reasons do people give for engaging in unhealthy behaviours? Have you ever done this? Why?
 - What kinds of reasons do people give for not engaging in healthy behaviours? Have you ever done this? Why?
 - What can you say to a family member whom you want to support, but who engages in unhealthy behaviours?
 - What can you say to a family member whom you want to support, but who fails to regularly engage in healthy behaviours?
 - How does this game help YOU to make healthier choices?
 - How does this game help you to help others make healthier choices?

Wrap-up

Review the session objectives. Answer any remaining questions.

Background information

People used to think that knowledge was enough to change behaviour, but scientists and programme managers realized that many people know what they should do, but they still do not do it.

By understanding the reasons why, we can determine the best way to help them adopt healthy change. This lesson is a type of barrier analysis to help understand why a certain group does the things that they do, even though they know that it is not healthy.

For more information:

Barrier analysis: barrieranalysis.fh.org/background/introduction.htm

Barrier analysis in Red Cross Red Crescent context: IFRC's Behaviour change framework, 2013

Supportive environments

Time required

60 minutes

Audiences

- Youth peer educators
- Youth
- Adaptable for adults

Overview

Learners work in groups to identify resources in their home and school community that support healthy lifestyles.

Learning objectives

At the completion of this topic, learners will be better able to:

- Demonstrate effective communication skills.
- Relate how to appropriately manage the consequences of NCDs on youth, family and friends.
- Explain the importance of supportive environments for healthy living.

Materials

- "Change the world" video at <https://www.youtube.com/watch?v=oROsbaxWH0M>
- Supportive environments poster
- Two flipchart sheets
- Permanent markers

Preparation

- Make one A3 copy of the supportive environments poster
- Prepare two flipchart sheets as detailed below:

Group 1: School setting			
Ways to create supportive environment in the school	Available resources	Role of the youth	Possible in my community?
Group 2: Community setting			
Ways to create supportive environment in the community	Available resources	Role of the youth	Possible in my community?

Process

1. Give a brief overview of the objectives and activities in this session.
2. Show the "Change the World" video at <https://www.youtube.com/watch?v=oROsbaxWH0M>.
Explain that the school community environment was open and had few barriers to allow the children to live healthier lives. The one barrier they had was time.

Ask: How did they address the barrier of time?
3. For older audiences, also show the supportive environments poster and explain the characteristics of a supportive environment for healthy lifestyle.
4. Divide the class into two groups. Assign one group to be the community group and the other group to be the school community group.
5. Ask each group to select three people in their group for the roles of a leader, a writer and a reporter. The leader is responsible for making sure that all instructions are followed and the work is completed within the allotted time. The writer will write all the outputs of the group on the flipchart paper. The reporter will report their group's work to the larger group.
6. Show them the pre-prepared flipchart sheets. Explain that in their groups, they will discuss and add to the chart:
 - Specific actions they can take to create a supportive environment in their assigned community
 - Specific resources that are needed to carry out these actions
 - How youth can specifically facilitate this process
 - If they feel that this is a possible action in THEIR community(ies)
7. Explain that they have 30 minutes to discuss and complete their flipchart sheets. Synchronize time and inform participants when the timer starts. Ask them to wrap up in the remaining five minutes then end the group discussion.
8. Group 1 will present their output to the class. Group 2 will have the chance to ask questions to group 1 and vice versa. After each group presentation, ask the other group: What do you think about the group's output? Do you have any additional comment?
9. Hold a plenary to discuss the process. Questions may include:
 - Why are supportive environments so important for healthy living?
 - What can youth do to build a healthy environment?

Wrap-up

Review the achievement of the learning objectives.

Background information

"Good health and wellbeing require a clean and harmonious environment in which physical, psychological, social and aesthetic factors are all given their due importance. The environment should be regarded as a resource for improving living conditions and increasing wellbeing."

- European Charter on Environment and Health, 1989

Source: http://www.euro.who.int/__data/assets/pdf_file/0005/215645/HEALTH-AND-THE-ENVIRONMENT-IN-THE-WHO-EUROPEAN-REGION-Creating-resilient-communities-and-supportive-environments.pdf?ua=1

"A healthy city is one that is continually creating and improving those physical and social environments and expanding those community resources which enable people to mutually support each other in performing all the functions of life and in developing to their maximum"

- Hancock, T. and L. Duhl. 1988

"Any city can start the process of becoming a healthy city if it is committed to the creation and preservation of physical and social environments which support and promote better health and quality of life for residents. Incorporating health considerations into urban development and management is crucial to healthy cities."

"Key features of a healthy cities project include high political commitment; intersectoral collaboration; community participation; integration of activities in elemental settings; development of a city health profile and a local action plan; periodic monitoring and evaluation; participatory research and analyses; information sharing; involvement of the media; incorporation of views from all groups within the community; mechanisms for sustainability; linkage with community development and human development; and national and international networking."

Source: http://www.alliance-healthycities.com/PDF/AFHC_Directory_2014/08Philippines_AFHC2014Directory_01112014.pdf

Telephone

Time required

30 – 40 minutes

Audiences

- Youth peer educators
- Youth
- Adults

Overview

Learners play the traditional game of Telephone, being asked to relay a message from one end of their group to another through whispers. This lesson helps learners to understand the importance of delivering an appropriate message across many audiences. Learners learn what makes a message effective for a particular group or audience.

Learning objectives

At the completion of this topic, learners will be better able to:

- Identify the four NCDs of cancer, diabetes, CVD and CRD.
- Identify the risk factors for NCDs.
- Apply appropriate techniques to positively influence peers.

Materials

- Telephone PowerPoint slides
- Six sticky notes, with a letter between A and G written on each. This activity is designed for 30–40 participants. If you have a smaller group, divide into fewer groups but maintain the number of people (four to five) in each group.

Process

1. Explain that they will now take part in an activity that will introduce them to the importance of delivering effective messages.
2. Ask everybody to get into groups of four to five participants each.
3. Explain to the groups that they will take part in an activity to understand the importance of engaging the community appropriately with effective messaging.
4. Everyone must put away paper and pens and pencils. They may not write anything unless specifically prompted to do so.
5. Ask for one person from each group to come out of the room to meet you. They may not bring paper or pencils with them.
6. Explain to all the messengers that they will play the telephone game. You will give each of them a slip of paper with a letter between A and G and then you will read the corresponding sentence to the messenger. If a messenger would like to hear it again they can say "operator", and you will repeat. You may only repeat the sentence once.

They may not write anything down. They must wait for you to give each messenger their message before going back into the room.

7. The sentences per team are:

Group A: NCDs are noncommunicable diseases and they include diabetes, cancer, cardiovascular disease and chronic lung disease. NCDs usually occur when a person consistently practises poor health behaviours like tobacco use, harmful use of alcohol, eating too many unhealthy foods and/or being physically inactive.

Group B: NCDs are diseases that CANNOT be spread through contact with other people or animals that are sick. NCDs are the number one killer in the world, with more than 63 per cent of all global deaths due to cardiovascular and chronic lung diseases, cancers and diabetes.

Group C: Cardiovascular diseases are a group of problems that occur when the heart and blood vessels work too hard to get blood through your heart and vessels, bringing oxygen to your body. Cardiovascular diseases are mainly caused by a blockage that prevents blood from flowing to the heart or brain. This blockage is often caused by tobacco use, harmful use of alcohol, eating too many unhealthy foods and/or being inactive.

Group D: Cancer occurs when a single cell in one part of your body starts to grow abnormally and then grows uncontrolled in your body. Tobacco use, harmful use of alcohol, eating too many unhealthy foods and/or being inactive can lead to many kinds of cancers.

Group E: Chronic respiratory diseases occur when your airway and/or lungs become sick and make breathing difficult. Chronic respiratory diseases can occur when you use tobacco, drink too much alcohol, eat too many unhealthy foods and/or inhale too much smoke when another person is smoking nearby or from the smoke from burning wood or from charcoal-burning stoves.

Group F: Diabetes occurs when your body cannot control the level of sugar in your blood. There are different types of diabetes, but the most common type is called Type 2 diabetes. People who have excess body weight, eat too much sugar and/or are physically inactive are at higher risk for Type 2 diabetes.

Group G: The message is blank.

8. After you have read the sentences to the messenger for each group, explain to them again that they will go back into the room and you will then give the full group the instructions.

Messengers may not write anything down between coming into the room and starting the activity. They may not talk to their group mates until prompted.

9. Bring the messengers back into the room.
10. Explain to the full group that they will play the game of "Telephone". Explain that you took the people out of the room and gave them a sentence that they now have in their head. The messenger will whisper what they heard into the ear of the person next to them. Each person may ask for the sentence to be repeated only once by saying the word "operator". After hearing the sentence, they then turn to the next person and whisper the phrase they heard in their ear. This continues until the last person in each group has received the message.
11. The last person should write down what they heard and not share it with anyone at the table until prompted by the facilitator.
12. Ask if there are any questions. When the groups are ready, let the activity begin.
13. As the groups do the activity, walk amongst the tables to ensure the rules are followed. When the phrase has gone through each member of the groups, remind the last person to write down what they heard and keep it private.
14. When all the groups have passed their message both ways, the point person should write down the response that they received through the telephone.
15. The trainer then:
 - Shows or reads each individual prompt on the screen for a group.
 - Prompts the last person in the group to share what they heard as a request or message.
 - Compares the two.

Lesson plan guidance

Telephone

16. Discuss with the group what happened. Some questions to consider:
 - Why did the messages not get through accurately?
 - Was there one group that struggled more than others? Why?
 - One group did not receive the initial information. What happened there?
 - How does ensuring the accuracy of the message improve the likelihood that Red Cross Red Crescent work is appropriate and responsive?
 - Did the original messages always make sense?
 - How can we misinterpret a message by relying on what we THINK people are saying?
17. Explain that these messages are ineffective because they are not appropriate and do not follow the rules for effective messaging. The three elements of effective messaging are:
 - Keep it simple
 - Communicate a benefit
 - Call to action
18. Model how you would modify one of the given messages to meet the three criteria. For example, read a sample “Before” message below and see how modification helped to make it a more effective message.

Before

To prevent early death, people should reduce the occurrence of preventable risk factors which are an aspect of personal behaviour or lifestyle that is associated with an increase in the occurrence of a particular disease or other health condition.

After

Reduce unhealthy behaviours by not smoking or drinking, eating five fruits and vegetables and being active for an hour every day to feel great and live a healthier life.

The “After”-message is simplified and it gives:
Call to action
Communication of benefit

19. Have each group work together to take their original telephone message and modify it so that it meets the three criteria of an effective message.
20. After 20 minutes, review each group’s message and rate its effectiveness.

Wrap-up

Review the learning objectives for the session and how the session helped them to begin to learn the content.

